

Graylands Campus Project

Forensic Mental Health Expansion — Mount Claremont, Western Australia

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PURPOSE

This paper is prepared for community members, media, and elected representatives. It documents a \$698 million state government decision to convert a residential suburb's hospital into Western Australia's only large-scale forensic mental health campus — made without public consultation and based on a business case its own independent reviewer said was not good enough.

All facts are drawn from publicly available government sources, parliamentary records, and credible media. All claims are independently verifiable from the sources listed in Annex D.

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Executive Summary

This paper documents a \$698 million state government decision to transform the Graylands Hospital site in residential Mount Claremont into Western Australia's only large-scale dedicated forensic mental health campus. The decision was made through a government-internal process between 2021 and 2023. No record of direct, formal consultation with residents, schools, or local government has been identified in the public domain at any stage.

The community's documented concerns fall into eight areas — five governance failures, one environmental and heritage concern, one unresolved community safety matter, and one social and economic due diligence gap. Each is grounded entirely in publicly available government sources, parliamentary records, and credible media reporting. Each is independently verifiable.

The question this paper raises. The central question this paper raises is not whether Western Australia needs forensic mental health capacity. It does, and these patients require treatment. The question is narrower and, on the public record, unanswered: whether this particular model — a tripling of forensic capacity, the removal of the general hospital function, and a legislated pathway to supervised community access — is safe at this particular site, within one kilometre of four schools and approximately 2,000 students.

Why it is unanswered. No published document answers it. No site-specific security assessment, school-interface risk assessment, or community notification protocol for the expanded campus has been identified in the public domain. The site was selected in 2021 on transport and accessibility criteria that did not include security or school proximity; the legal framework governing what the facility must do changed in 2023, after selection; and the taskforce that could have re-tested the site against that new model dissolved in July 2023. This is not the first time these exact gaps have been raised: in 1990, before the site's first purpose-built secure unit was even built, a sitting MP made substantially this same case to the Legislative Assembly, and was not answered with a published assessment then either (Section 3, Section 8.6).

What this paper does and does not claim. This paper does not assert that the expanded facility will be unsafe. That claim would require comparative outcome evidence which does not exist in published form. It asserts something more limited and more difficult to rebut: that on the current public record, neither this community nor the government can demonstrate that it is safe — and that a \$698 million irreversible decision adjacent to four schools should not proceed to Stage 1 construction while that is true.

Concern 1 — Community safety protocols are unresolved and publicly undisclosed (Section 3)

This paper documents five specific, evidence-based risk factors, not a general objection to forensic care: absconding frequency, transition-related failures, release and diagnostic misjudgement, compliance-dependent leave models, and a legislated push toward community access. None of this is new: in November 1990, before the site's first secure forensic unit existed, a sitting MP told the Legislative Assembly this exact failure pattern was inevitable, citing incidents already on the record at that time. It recurred anyway, repeatedly, over the following three decades. In the only year with a published facility breakdown, this hospital's general wards recorded seven times the absconding rate of the next-highest WA facility — the hospital's own secure forensic unit is recorded separately, at under 5 (Section 3). The most serious documented outcome is a 2018 case in which a diagnosis a court-appointed psychiatrist later found to be wrong preceded a homicide, two days after the patient absconded.

Concern 2 — The site was chosen on criteria that never considered security (Section 4)

Infrastructure WA's own report confirms that twenty sites were assessed for this facility in 2021, using criteria limited to accessibility, central location, and proximity to public transport. None of the twenty sites were assessed against community safety, school proximity, or security risk.

Concern 3 — The law changed after the plan was made (Section 5)

The Criminal Law (Mental Impairment) Act 2023 replaced the indefinite detention model with mandatory community reintegration as the primary purpose of forensic facilities. It received Royal Assent in the same month as the \$218.9M funding commitment.

Concern 4 — The government's own independent advisor said the business case was not good enough (Section 6)

Infrastructure WA — the State's statutory independent advisory body — formally assessed the business case and found it contained 'insufficient information on which to base an investment decision,' citing six specific deficiencies. The government committed \$218.9 million approximately three weeks later.

Concern 5 — The previously indicated public direction was reversed without explanation (Section 7)

The State's own Mental Health Plan 2015–2025 described the Graylands institutional model as a 'Victorian era asylum model' — implying replacement, not expansion. The reversal to \$698M forensic expansion on the same site was never publicly explained.

Concern 6 — The community was never consulted (Section 8)

Over five years of planning for a \$698M project adjacent to four schools and approximately 2,000 students, no formal community consultation was conducted with residents, school communities, or local government. Government has done this differently before, at this exact site: in 1990 residents petitioned Parliament for consultation before any decision, and in 1993 government ran a genuine public process before opening the smaller Frankland Centre. Both the scale of what's proposed now and the standard government has already met here make the current silence harder to justify, not easier.

Concern 7 — Threatened species habitat and heritage values are at risk (Section 9)

A referral under the Commonwealth EPBC Act, No. 2025-10271, confirms two federally listed threatened cockatoo species are assessed as likely to occur in the proposed action area. The Graylands campus is listed on the WA State Register of Heritage Places (No. 13630).

Concern 8 — No published case that Graylands, at this scale, is the right long-term model (Section 10)

WA's own psychiatrists say the state needs more forensic mental health capacity, urgently — and this report agrees. But the profession that would staff it does not recommend meeting that need with one large campus.

Stage 1 (\$218.9M, 40 new beds) is funded and in design. The full 136-bed program (\$698.3M) has not been separately funded. Stage 2 funding approval is the community's primary leverage point for demanding accountability before further irreversible commitments are made.

The community is asking the Cook Government to pause all site works, release the full evidence base, and conduct genuine community consultation before Stage 2 is funded. This paper provides the documented factual basis for that request.

1. The Project: What Is Actually Planned

1.1 Background and the Critical Distinction Between Patient Populations Graylands Hospital at Brockway Road, Mount Claremont has operated on its approximately 10-hectare site since 1909. Its predecessor, Claremont Hospital for the Insane, was sited here from 1903 because the location was "healthy, accessible to the city, but far enough from built-up areas not to cause distress or offence to susceptible citizens" (Bolton and Gregory, cited in Mikus, 2013, p.36) — the opposite of its position today. It is listed on the State Register of Heritage Places (Place No. 13630) and has served as Perth's main standalone public psychiatric teaching hospital. The hospital has historically accommodated two distinct patient populations. Understanding the difference between them is essential to understanding what the proposed expansion means for the surrounding community. General psychiatric patients are people experiencing serious mental illness — conditions such as schizophrenia, severe depression, or bipolar disorder with psychotic features — who require inpatient care. This includes both voluntary patients and those admitted involuntarily under the Mental Health Act 2014 for their own safety or the safety of others. The majority have not been charged with criminal offences. Their admission is governed by health legislation, not criminal law. Forensic mental health patients are an entirely distinct legal category. They are individuals who have been charged with serious criminal offences and have entered the forensic mental health system because a court has found them either: (a) not fit to stand trial due to mental illness; or (b) not criminally responsible for their offending due to mental illness at the time it occurred. The offences involved are typically serious. At WA's forensic facilities, they include charges of homicide, manslaughter, grievous bodily harm, serious sexual offences, and armed robbery. Detention is under a court order administered through the Criminal Law (Mental Impairment) Act 2023 — not a standard health admission. The clinical program manages the patient's mental illness in a secure environment and, under the new law, supports eventual reintegration into the community under supervision. The Frankland Centre has operated at Graylands since 1993 as WA's maximum-security forensic mental health unit — housing patients whose offending history and clinical presentation require the highest level of secure management within the forensic system. It is not a general psychiatric ward with additional locks. It is a purpose-built maximum-security facility for patients charged with the most serious categories of criminal offence. Total current forensic inpatient capacity across the Frankland Centre (30 beds) and the adjacent Dryandra Ward (15 beds) is 45 beds. The proposed expansion converts Graylands from a general psychiatric hospital with a contained forensic wing into a campus designed primarily around the forensic population. The general psychiatric hospital function will be substantially closed or relocated. Forensic patients arrive at the facility through two pathways. Some are transferred from prison when their mental illness becomes too severe to be managed in a correctional setting. Others come directly from the courts after being found by a judge either not fit to stand trial due to mental illness, or not criminally responsible for their offending. Either way, they are patients whose mental illness is directly connected to serious criminal conduct and who require a level of secure management that reflects their assessed risk. Graylands is not the only facility in Western Australia that holds people assessed at this level of risk — but it is the only one in a residential suburb. Western Australia's maximum-security corrective services facilities, including Casuarina and Hakea prisons, are located in Canning Vale in Perth's southern suburbs — an industrial and correctional precinct well separated from residential areas, homes, and schools. Graylands, surrounded by residential estates on three sides and approximately 270 metres from a school of 1,500 students, is the exception in WA's estate of secure facilities. The community has never been given a public explanation of why this site was chosen and retained for major expansion over alternatives that do not carry this residential proximity. Most residents and school parents remain unaware that this project is underway. The site decision was made in 2021. The project was funded in 2023. A contractor was engaged for the main campus design in April 2026. At no point in that five-year period was a single public information

session held in Mount Claremont. No direct written notification was sent to residents or to the parent communities of the four nearby schools. The relocation of the Claremont Therapeutic Riding Centre to a new

facility at 9 John XXIII Avenue, directly opposite John XXIII College, is scheduled to proceed ahead of the main site works — the Government's stated aim, as at April 2026, is to complete that transition during 2027 — and no prior community notification has been recorded.

1.2 What Stage 1 Delivers

Stage 1, funded in the April 2023 State Budget at \$218.9 million (including \$15 million Commonwealth), will deliver, on the northern campus: • 32 new sub-acute male forensic mental health beds • An 8-bed child and adolescent forensic mental health unit — the first dedicated facility of this kind in Western Australia • An Integrated Patient Services and Rehabilitation Centre (Hub) • Secure partial perimeter and civil infrastructure works Adding these to the existing 45 forensic beds (30 Frankland Centre + 15 Dryandra Ward) will bring the total to approximately 85 forensic inpatient beds on the northern campus after Stage 1. What 'Sub-Acute Forensic' Means for the Neighbourhood The 32 new beds are classified as 'sub-acute' forensic — a lower security classification than the existing Frankland Centre maximum-security facility. Sub-acute forensic patients are individuals who remain under forensic custody orders but are at a stage of recovery where maximum-security containment is not required. They participate in structured community access programs — escorted or semi-supervised outings beyond the campus perimeter — as part of their rehabilitation pathway. The boundaries and protocols for these programs have not been publicly disclosed.

1.3 The Long-Term Plan: 136 Forensic Beds and \$698 Million

Government communications about this project focus on Stage 1. The full plan, confirmed in Infrastructure WA's publicly available Major Infrastructure Proposal Assessment (March 2023), is substantially larger: Long-Term Campus Plan — Confirmed in Infrastructure WA Major Infrastructure Proposal Assessment (MIPA), March 2023 136 forensic mental health beds on the northern campus

~3× current forensic capacity

40 secure mental health recovery and rehabilitation beds

Additional clinical capacity

Closure of 118 existing general psychiatric beds (51 acute + 67 HECS beds)

General psychiatric function substantially closed

Demolition of Fitzroy House and Murchison House

Significant campus restructure

Estimated total capital cost

\$698.3 million

Graylands Hospital — historically serving general adult psychiatric patients — will be fundamentally transformed into a predominantly forensic campus. The general psychiatric hospital function will be substantially relocated or closed.

1.4 The School on the Boundary

John XXIII College sits on the immediate boundary of the Graylands site — approximately 270 metres from the northern construction zone and directly opposite the CTRC relocation at 9 John XXIII Avenue. It is not one of several distant schools in the general area. It is the school the works are happening next to, and it holds roughly 1,500 students, from Pre-Kindergarten to Year 12. The College's own published history records that its 24.6-

hectare campus was purchased directly from land the State Government held as part of Graylands Hospital, at a time the Government was reducing its footprint on this site — and that the campus was built, deliberately, without a security perimeter: “The campus, with its open design, accessible grounds, and lack of perimeter fencing, was itself a statement about the kind of

community the College intended to be... The campus was never fenced. It was never meant to be.” Students arrived at this site on 12 May 1986. The sequence that matters John XXIII College has occupied this open, unfenced site since 1986. The Frankland Centre — the maximum-security forensic unit now being expanded — did not exist until 1993, seven years later. Every subsequent escalation at this site has occurred after the school was already there: the Dryandra Ward conversion, the 2021 Taskforce recommendation, the 2023 funding commitment, the CLMI Act's community reintegration mandate, and the current construction phase. The school did not move in next to a forensic facility. The forensic facility, and everything built on top of it since, arrived next to an open, already established school. Four further schools sit within about a kilometre of the campus: School

Distance / Notes

Students (approx.)

Mount Claremont Primary School

~550–750m; government primary; formerly Graylands Primary School (since 1917)

~300 (K–Year 6)

Moerlina School

Within immediate residential catchment; independent primary

Independent primary

Quintilian School

Within immediate residential catchment; independent primary, 46 Quintilian Road

Independent primary

Shenton College

~1km; government secondary

~2,800 (Years 7–12)

Approximately 4,800 students attend school within about a kilometre of the Graylands campus. The northern construction zone — where the new forensic beds will be built — is the section closest to all five, and closest of all to John XXIII College's open, unfenced boundary.

1.5 Stage 1: A Fundamental Change, Not an Incremental Addition

A commonly heard position is that Stage 1 represents a modest extension of what already exists at Graylands, and that Stage 2 — with the full 136-bed campus — is where the more consequential decisions lie. That position contains factual errors that matter for how the community and its representatives should respond. What Stage 1 already changes Three aspects of Stage 1 make it a categorical change rather than an incremental one. First, the 32 new sub-acute beds are fundamentally different in purpose from the existing Frankland Centre. The Frankland Centre is a maximum-security containment facility. Sub-acute beds are not. Patients at this classification remain under forensic custody orders but are at a stage where maximum-security management is no longer clinically

required. Their clinical program is built around structured community access — escorted and semi-supervised outings in the surrounding streets of Mount Claremont — as part of the structured community-access pathway the CLMI Act 2023 was designed to expand, authorised case by case rather than automatically. The Frankland Centre confines people. The sub-acute wing moves patients progressively through the surrounding suburb. Second, the 8-bed child and adolescent forensic mental health unit is an entirely new institution — WA has never had one. It is being built on the northern construction zone approximately 270 metres from John XXIII College. Under the CLMI Act's Children's Court forensic jurisdiction (from September 2024), children and adolescents at this unit are on the same community reintegration pathway as adult forensic patients. No school-interface safety assessment for this unit has been publicly released. Third, Stage 1 nearly doubles the existing forensic bed count from 45 to approximately 85. Under the old indefinite detention regime, the forensic population was largely stable and long-stay. Under the CLMI Act's

limiting terms, it is a higher-throughput population — patients cycling through stages, transitioning between security levels, and spending increasing time in the surrounding community. The community interface of 85 beds operating under the CLMI Act is structurally different from, not merely larger than, 45 beds under the old law. The CLMI Act is the critical variable. The Government may argue that Stage 1 is a modest addition to a facility the community has lived alongside for decades. That argument was plausible under the old law, when forensic patients were largely confined. Under the CLMI Act — in force from September 2024 — community reintegration is not exceptional. It is a statutory purpose. Stage 1 is not being designed into the old legal landscape. It will operate in the new one.

Understanding the ECI contract — and what it does not mean The engagement of ADCO Constructions in April 2026 under an Early Contractor Involvement (ECI) contract is sometimes characterised as placing the project beyond the point of review. This misunderstands what an ECI contract is. ECI is a standard procurement mechanism in major public construction, specifically designed to engage a contractor during the design phase — before a full construction contract is awarded — to provide cost modelling, buildability input, and programme refinement. It is not a construction contract. Main campus construction has not commenced. The Heritage Impact Statement has not been submitted. The City of Nedlands Development Application has not been lodged. The full construction contract has not been signed. The ECI phase is, by design, the stage at which scope and cost are still being refined before commitment to build. Sunk cost reasoning does not apply here. The \$218.9M is a budget allocation from 2023 — it is not spent construction money. As at August 2026, no physical construction has commenced on the main campus or at the CTRC relocation site; contractor procurement for the CTRC relocation was reported as still underway in April 2026, with the Government targeting completion of that relocation during 2027. A design-phase contract and a not-yet-commenced ancillary relocation do not constitute grounds for proceeding to full construction without addressing the documented governance gaps. No amount already committed changes the validity of those gaps. Applying sunk cost logic to public funds is not fiscal responsibility — it is a mechanism for compounding inadequate decisions with irreversible ones.

The window that still exists The formal processes ahead of the full construction contract are not obstacles. They are the scrutiny mechanisms that exist precisely for decisions of this scale. The Heritage Impact Statement requires submission to and assessment by the Heritage Council of Western Australia. The City of Nedlands Development Application triggers a statutory public notification and submission period. Neither has occurred. Stage 2 — the path to 136 beds — has not been separately funded. Whether Stage 1 as a standalone 85-bed facility on this site, adjacent to four schools and operating under the CLMI Act's community reintegration framework, represents the appropriate outcome of a \$218.9M public investment is a question a genuine evidence base and genuine community consultation should have answered. That process has not occurred. The window is not indefinite. The Commonwealth referral itself estimates construction beginning 4 June 2026 and running

through to 28 April 2029. The Heritage Impact Statement and the City of Nedlands Development Application sit ahead of that date, not behind it — this is the period in which they can still happen meaningfully.

2. How the Decision Was Made

2.1 The Prior Publicly Indicated Direction Before the Graylands Reconfiguration and Forensic Taskforce was established in January 2021, two government documents had established a publicly indicated direction for the Graylands site that pointed toward replacement rather than expansion. The WA Mental Health, Alcohol and Other Drug Services Plan 2015–2025 — the government's own ten-year strategic plan for mental health services — described the Graylands institutional model as based on a 'Victorian era asylum model.' This language signals that the government's own planning framework regarded the model as outdated and in need of replacement with contemporary, community-based services, not expansion in situ. In the 2019 State Budget, the Department of Health received \$3 million for planning and decommissioning studies of the Graylands site. Funding for studies of this type is consistent with a direction of preparing for site closure and service relocation, not preparing for a \$698M expansion on the same site. Neither document explicitly committed the government to closing the Graylands facility. However, the direction of travel they established was sufficiently clear that when major forensic expansion on the same site was announced in 2023, the then-Shadow Minister for Health described it publicly as an 'about-face' at odds with the government's own plan. That characterisation has not been publicly contradicted by the government with documentation. The decision that reversed the prior direction — the establishment of the Taskforce in January 2021 with a mandate to consider 'reconfiguration' rather than replacement — was never publicly framed as a change in strategic direction. The Taskforce's terms of reference have not been published. The community has never been told when the direction changed, by whom it was authorised, or what analysis supported it.

2.2 The Taskforce

In January 2021, the State Government established the Graylands Reconfiguration and Forensic Taskforce (GRAFT), chaired by the Hon. Jim McGinty AM — former Minister for Health and former Attorney-General — to advise on the future of the Graylands site. The Taskforce comprised six senior public servants and one independent member. Structural anomaly in Taskforce composition The Taskforce did include Department of Justice representation, appropriate given the forensic mental health legal framework falls under joint Health-Justice administration and the Criminal Law (Mentally Impaired Accused) Act 1996 at the time required the Attorney-General's personal approval for patient releases. What it did not include, at any stage, was a seat for community or local government representation. The CLMI Act reform — which would fundamentally change that framework — was also being developed during the Taskforce's term (2021–2023). Whether formal inter-departmental coordination between the Taskforce and the CLMI Act reform process occurred, and what it produced, has not been publicly disclosed. By comparison, Victoria's equivalent forensic mental health provider, Forensicare, is formally structured to report to both the Minister for Mental Health and the Minister for Corrections — a dual-portfolio governance arrangement that reflects the joint health-justice nature of forensic mental health service delivery. The Graylands Taskforce did include Department of Justice representation, but only for the life of a taskforce that ran from 2021 to 2023 and was then dissolved. Victoria's dual-portfolio reporting line for Forensicare is a permanent structural feature of how that state governs forensic mental health, not a temporary arrangement tied to one project's planning phase. The governance footprint has fragmented further since. Delivery of the Graylands construction program now sits with the Office of Major Infrastructure Delivery (OMID), which — following a machinery-of-government reform effective 1 July 2025 — is now part of the Department of Transport and Major Infrastructure, having previously operated as a health-specific delivery body. Clinical and service planning sits with the North Metropolitan Health Service (NMHS) and the Mental Health Commission; the legal framework sits with the Attorney-General's portfolio; the funding decision sat with Treasury and Cabinet. No single one of these

bodies has a mandate to ask whether the whole plan, taken together, still makes sense. A government tender published in May 2026 confirms OMID's own role is delivery — its project team engages engineering advisers to

oversee construction, acting 'in consultation with' Health and NMHS — and that the project has continued to move through internal Treasury-framework processes since: an updated business case was subsequently approved, and in November 2025 Cabinet approved a Project Definition Plan, which the same tender describes as refining the project's 'strategic justification and objectives' and delivering 'value for money.' That is a scope-and-cost exercise. Nothing published from it addresses site suitability, the CLMI Act's changed operating model, or the schools and homes on the boundary (Annex B). Why this matters in practice A question like “who decides the boundary of a supervised leave route past a school” does not sit cleanly inside any one agency's job description. Health can reasonably say that is a security and planning matter. OMID can reasonably say it delivers buildings, not clinical protocols. The Department of Justice can reasonably say leave conditions are set by the Tribunal, not by government departments. Each answer is individually defensible. Together they mean the question can go unanswered indefinitely, because no single agency is responsible for making sure it gets answered at all. This is not a claim that fragmentation was designed to avoid accountability. It is a claim that fragmentation has that effect regardless of intent, and that the community has no way of knowing who to approach with a schoolsafety question, a construction complaint, or a request for the community access boundaries once the facility is operating. The government has not named a single senior official accountable for the project as a whole, nor a single point of contact for resident and school concerns once construction and operations begin. Both are reasonable, specific things to ask for — and neither requires resolving which department should ultimately own forensic mental health infrastructure. It only requires the government to say, in public, who answers when something goes wrong.

2.3 The Critical Three-Event Sequence: March–April 2023

Three events occurred within approximately four to six weeks of each other in March–April 2023. Each event is documented in publicly available sources. The sequence has never been publicly explained: March 2023 — Infrastructure WA MIPA: business case has 'insufficient information on which to base an investment decision' March 2023 — Criminal Law (Mental Impairment) Act 2023 passes the WA Parliament 13 April 2023 — CLMI Act 2023 receives Royal Assent. New law formally enacted. April 2023 (~3 weeks after IWA finding) — Government commits \$218.9M in Stage 1 funding in State Budget 28 June 2023 — A contract to actually develop that business case and finalise costings is awarded to Ernst & Young, two months after the funding was provisioned No public record exists explaining how these events — the insufficiency finding, the new legal framework, the funding commitment, and the business-case contract that followed — were sequenced and coordinated.

2.4 Key Decisions — Dates, Actors, and Transparency

Date

What Has Been Released

What Has Not Been Released

Site selected for forensic expansion (northern campus)

Oct–Nov 2021

Ministerial statement to Parliament Nov 2021

Cabinet submission; decision trail; basis for site selection over alternatives

Government formally

Nov 2022

Description in IWA MIPA (published Sep

Cabinet papers; formal

Decision

endorses expansion

2023)

approval documentation

\$218.9M committed in State Budget

Apr 2023

Budget announcement

Formal response to IWA concerns; updated business case; interagency sign-off chain

ADCO Constructions engaged (Early Contractor Involvement)

9 Apr 2026

Government media statement

ECl contract scope; design parameters

3. Concern 1 — Community Safety: The Documented Record and Unresolved Questions

The following incidents are drawn from credible published sources: WA parliamentary records, the State Coroner, mainstream media, and — because this is not solely a question about one WA facility's history — the official record of a comparable Australian forensic mental health system operating under a more mature version of the same community-reintegration model the CLMI Act 2023 now introduces here. The case for expanded and modernised forensic facilities partly rests on acknowledging this history. That same history gives the community documented, evidence-based grounds to demand rigorous safety planning and transparent community notification protocols before expansion proceeds.

The burden this record establishes. This paper cannot prove that harm will occur under the proposed model. No one can — the model has not been operated at this scale, at this site, and no comparative outcome data exists for it. That is precisely the point being made. The government has not published the assessment that would let anyone, this community or an independent reviewer, check that prediction either. The burden this record places on the government is not to disprove a forecast. It is to publish the evidence that would let the forecast be tested — evidence that, on the current public record, does not exist.

The incidents, statistics and comparable cases set out in the rest of this section are not presented as a miscellany. Read together, they describe five specific, documented risk factors. Each is drawn from the record below, not asserted in the abstract — the 'Why It Matters' column in the table at 3.3 links every individual incident back to the risk factor it illustrates.

Risk factor 1 — Absconding is frequent, and the public record shows what can follow it

In the only year for which a full facility breakdown has been published, Graylands' general wards recorded 153 absconding events — seven times the next-highest hospital in the state (3.4) — while the same document lists Frankland Centre, the hospital's existing secure forensic unit, separately at fewer than 5. Government does not publish what became of the other 152. The distinction matters for what Stage 1 actually adds: its new 32-bed sub-acute unit is a lower security classification built around structured community access — closer in design intent to the general wards behind the 153 figure than to Frankland's own tighter model. What the incidents at 3.3 show is what a small fraction of them led to when they became public — and the earliest of those incidents was disclosed to Parliament in 1990, six years before the purpose-built Frankland Centre even opened (see below).

Risk factor 2 — Transitions between security levels are where documented failures cluster

Two of the clearest failures in this record — a documented pattern of repeated unauthorised absence in 2010, and a 2014 case involving a move between Frankland Centre and a lower-security step-down facility — both occurred specifically during a transition between security tiers, not while a patient was static on one ward (3.3). No published document explains who decides a patient is ready to move tiers, what happens when that judgement is wrong, or how often the CLMI Act's legislated reintegration pathway (Section 5) now requires that decision to be made. This exact mechanism was described to Parliament in 1990, before it had ever been observed in practice at the purpose-built facility — see below.

Risk factor 3 — Release and diagnostic judgements have gone wrong, with fatal consequences

The clearest example in this record (3.3, Dec 2018) involved a diagnosis a court-appointed psychiatrist later testified was wrong, followed by a homicide two days after the patient absconded. This is not a claim that clinicians act other than in good faith — diagnosing psychosis is genuinely difficult. It is the claim that when this specific kind of judgement is wrong, the cost falls on someone outside the facility who had no part in making it,

and no document has been published showing how that risk is managed for a campus of this scale, this close to four schools.

Risk factor 4 — Some leave models depend entirely on the patient's own compliance

One case in this record (3.3, Feb 2017) shows a security downgrade, based on clinical improvement, that granted day-release privileges conditional on the patient returning to Graylands each night — and, in between, on his own self-enforced compliance with medication. Police explicitly tied the danger he posed to that compliance breaking down. A second, independent case the same year shows the identical pattern. Nothing in the public record describes what, if anything, monitors that compliance while a patient is in the community rather than in the facility.

Risk factor 5 — The law replaced indefinite detention with a mandatory review timetable that pushes toward community access

The Criminal Law (Mental Impairment) Act 2023 (Section 5) replaced indefinite detention with mandatory limiting terms and made community supervision a standard outcome rather than an exception. This is not a criticism of that reform — moving away from indefinite detention is broadly regarded as appropriate, and people held under these orders are entitled to the least restrictive setting consistent with safety. What has changed is structural: every custody order now sits inside a mandatory review timetable, and the Act's mechanisms for supervised leave and reintegration are substantially wider than before — though any individual leave decision is still made case by case, weighing the risk to community safety, under the Mental Impairment Review Tribunal. No published document shows that changed structure has been assessed for a site tripling in size next to five schools.

THIS WAS FORESEEN — THE 1990 PARLIAMENTARY WARNING

Every risk factor above was put to the Western Australian Parliament in detail, on the record, before the site's first purpose-built secure forensic unit — the Frankland Centre — was ever built. This is not a general warning read in hindsight. It is a specific, dated, sourced parliamentary record that named the mechanisms above one by one and asked government to act before construction, not after an incident.

On 8 May 1990, the then-member for Cottesloe, Bill Hassell, tabled a 646-signature community petition. It asked government to abandon the proposed forensic unit outright, or at minimum to fully discuss and justify any future version to the community before a decision was made — not after one (Hansard, Legislative Assembly, 8 May 1990, p.320).

Hassell resigned that June. The by-election sent Colin Barnett to the seat of Cottesloe — Barnett went on to serve as Premier of Western Australia from 2008 to 2017. Within months of taking his seat, on 29 November 1990, Barnett gave the Legislative Assembly a detailed, sourced case against the plan (Hansard, Legislative Assembly, 29 Nov 1990, pp.8173–8177):

- Two broken assurances, both on the record. When the neighbouring Swanbourne Hospital — which had held the site's forensic patients — closed in 1983, a resident named in Hansard as Mrs Jones wrote to the Health Department's Mental Health Services division, worried that people with criminal histories would end up at Graylands instead. The Director of Mental Health Services, Mr F. Bell, replied in writing that the closure would not affect “the nature of the patient population at the Graylands Hospital” — the two hospitals, he said, were separate entities. Within a few years, some of the patients that letter said would not come were being held at Graylands in what Barnett told Parliament was very low security. John XXIII College had relocated to this exact site the previous year on the same kind of assurance, given in various forms by government.
- Absconding harm already on the record, years before the secure unit existed. Barnett told the House that a Graylands patient in low-security care had left the hospital and, in Perth's CBD, sexually assaulted a young girl in 1987. He said three further people he considered dangerous had absconded from Graylands in 1990 alone, one having recently committed a serious stabbing.

- The transition-failure mechanism, named and predicted. Barnett told Parliament that moving offenders progressively from high to low security carried a risk that could not be engineered away, and that further offences against civil patients, residents or children were inevitable, not hypothetical, without a physical buffer at the point of first transition. No such buffer was built, then or since.
- The objection pre-empted the label it would later be given. Barnett described the opposition as “a very reasoned and responsible opposition coming from informed people,” distinct from panic — the same characterisation this campaign has had to make for itself, 36 years later.

Barnett also put an unpublicised detail on the record that resists any simple residents-versus-patients framing: the mother of one of the state's most serious offenders had phoned him — not to attack him, but to say she hoped her own son would not be placed in a residential-area unit either, because the attention would make his rehabilitation harder.

Barnett's own remedy was a properly buffered, purpose-built facility in an isolated location, whatever that cost. Government built neither the buffer nor the isolation. The specific failure mode he described recurred in 2004, 2010, 2012, twice in 2014, and 2015 — set out in full at 3.3 below.

Government's own words test this against the present. Barnett was Premier in October 2014 when a Frankland Centre patient failed to return from approved leave — the exact transition failure his 1990 speech had described. His response as Premier was that the process had been handled properly, adding: “I'm sure he'll either turn up or be found” (ABC News, 7 Oct 2014; full detail at 3.3, Oct 2014 row). The standard he demanded of government in opposition was not the standard he applied in government when the mechanism he had named actually failed.

Sources for this section: Hansard, WA Legislative Assembly, 8 May 1990 and 29 November 1990 (full transcripts held on file and available on request); ABC News, 7 October 2014.

3.1 Three Cases That Show the Range

Every incident in the table at 3.3 supports the risk factors above. Three cases do more than support them. Together, they show the full range of what these risks mean in practice: the most severe outcome on record, the clearest illustration of how the leave and release mechanism actually works, and the one case in this entire record where government responded the way this paper is asking Western Australia to respond now.

THE MOST SEVERE OUTCOME IN THIS RECORD — DECEMBER 2018

Jesse de Beaux

Graylands diagnosed Jesse de Beaux with drug-induced psychosis and treated him on that basis. The court-appointed psychiatrist who examined him after the killing testified the diagnosis was wrong: de Beaux ‘clearly had a psychosis in the absence of substance abuse’ and ‘was not in control of his actions.’ He absconded two days after leaving Graylands’ care and fatally stabbed a Munster woman, a stranger to him, in her own home. The WA Supreme Court found him not guilty by reason of unsound mind; Justice Fiannaca identified ‘lost opportunities’ to properly diagnose him, including at Graylands. He remains under a custody order, subject to the same limiting-term and review requirements introduced by the CLMI Act 2023 (Section 5).

Why this is one of the three: it is the most serious documented outcome in this record, and it demonstrates Risk 3 as plainly as the public record allows — a good-faith clinical judgement, later found wrong, with the cost falling on someone who had no part in making it.

THE CLEAREST ILLUSTRATION OF HOW THE MECHANISM ACTUALLY WORKS — FEBRUARY 2017

David Lockyer Harman

David Lockyer Harman, a convicted rapist with a history of violent offending, spent several weeks as the only patient in a secure six-bed ward, monitored by security guards 24 hours a day. When his condition was assessed as improved, his security classification was downgraded and he was moved to a less secure part of the campus — a decision that also granted him day release, letting him move freely in the community on the condition that he returned to Graylands each night for treatment and assessment. He failed to return. Police said he could become violent because he was not carrying the medication he needed to manage his condition, and warned the public not to approach him. He was located more than 24 hours later.

Why this is one of the three: unlike de Beaux, nothing tragic resulted — which is exactly why it matters. It shows the mechanism this expansion will run at nearly three times the current scale, in its routine, undramatic form: a clinical downgrade decision (Risk 3) unlocks community access that then depends entirely on the patient's own, self-enforced compliance (Risk 4), with no external check described anywhere in the public record.

THE CLOSEST WA HAS COME TO AN ADEQUATE RESPONSE — AND ITS UNRESOLVED AFTERMATH, 2015–25

Bennett Brook Disability Justice Centre, Caversham

Bennett Brook, Western Australia's only other 'declared place' for mentally impaired accused, had two residents escape on New Year's Eve 2015. Lockridge residents said what Mount Claremont residents are saying now: the centre was too close to homes and a primary school, and they deserved to be told about escapes. Government did nothing after the first two escapes. It took a third, in May 2016, before Disability Services Minister Helen Morton — the same Minister a Mount Claremont resident wrote to in February 2013, and appears never to have heard back from — commissioned an independent review and funded \$640,000 in security upgrades. Even that has not settled the underlying problem: the Mental Health Advocacy Service's own 2024–25 report, tabled in Parliament, records the centre as still 'under-utilised,' exactly the finding Hon. Alison Xamon MLC made in 2020.

Why this is one of the three: it shows both halves of this paper's argument in one place. Government can respond adequately — eventually — but only did so after the third documented failure, not the first, and even that response has not fully resolved community confidence nine years later. That is the strongest case WA has for how this could go if handled well, and it still took repeated harm to get there. Full detail, and how this same pattern repeats interstate, is set out in Annex A.

3.2 The Full Documented Log, 1987–2025

The table below lists every incident referenced above, in date order, with what each one establishes and its original source. The 1987 and 1990 entries are drawn from the parliamentary record described above; every entry from 2004 onward is independently corroborated by contemporaneous media, coronial, or parliamentary sources as cited.

Date	What Occurred	Why It Matters	Source
1987	A Graylands patient in low-security care left the hospital and, in Perth's CBD, sexually assaulted a young girl. Colin Barnett — then the newly elected member for Cottesloe — put this on the record in Parliament three years later as an example the government already knew about.	Risk 1 — The earliest documented absconding-related harm on this site now on the public record — six years before the purpose-built Frankland Centre existed.	WA Parliament Hansard, 29 Nov 1990

1990	Barnett told the Legislative Assembly that three people he considered dangerous had absconded from Graylands that year alone, one of them having recently committed a serious stabbing.	Risk 1 — Multiple absconding incidents in a single year, read into the parliamentary record while government was still deciding whether to build a secure unit at all.	WA Parliament Hansard, 29 Nov 1990
Late 1990s-2000	A pattern of inpatient deaths at Graylands. The ratio of suicides to admissions rose from ~2 per 1,000 (1987) to 6.6 per 1,000 (1999), with multiple cases showing failures of perimeter security and observation.	Baseline — Perimeter and observation failures at this site predate every incident below — this isn't a recent problem.	WA Parliament Hansard
Sept 2004	Two Frankland Centre patients, one with convictions for violent offences, smashed through a wall and jumped the security fence of a unit the Attorney-General had called "escape-proof" — it had reportedly been tested by the SAS. Police called them "extremely dangerous" and ran land and helicopter searches.	Risk 1 — Undercuts the premise that "maximum security" classification alone is sufficient — a unit built and tested to be escape-proof was defeated.	ABC News, 18 Sept 2004
Sept 2007	Manuel Francisco, released from Graylands five days earlier, attacked mental-health nurse Christine Bruce with a sharpened screwdriver as she walked to work, inflicting 24 stab wounds. He was convicted of attempted murder.	Risk 3 — A release-timing judgement proved wrong within five days, with the harm falling on someone with no connection to the decision.	ABC News
2010	Gregory Hunt, a long-term involuntary patient, was moved between locked and open wards repeatedly, including two unauthorised overnight absences — the first ended when police returned him the next evening; the second when he returned on his own.	Risk 2 — The clearest example on record: each absence coincided with a transition between security tiers, not a static placement.	WA Coroner's Court
July 2012	David Charles Batty held his 79-year-old father hostage in a car at Kings Park for more than four hours, threatening police with a gas cylinder and lighter. Found unfit to stand trial, he was detained at the Frankland Centre — from where he would later abscond twice.	Background — Shows how someone enters this system — and previews Risk 2: he'd go on to abscond during a security-tier transition, twice.	Newcastle Herald (AAP)
Dec 2012	A patient walked out undetected. Two days later, police brought in an unrelated homeless man matching a general description, and staff administered Clozapine — an	Risk 1 — The response to an absconding event caused serious harm to an uninvolved member of the public — a consequence no published protocol addresses.	The West Australian

	antipsychotic with a risk of fatal reaction — without confirming his identity. A subsequent audit found 12 patient bedrooms and 16 clinical rooms with no CCTV coverage.		
15 Feb 2013	A Mount Claremont resident reported a man — identified to him on the scene by police and hospital security as a Graylands patient — attempting to force entry into a neighbouring home, then moving through several more properties before police arrived.	Risk 1 — A direct, named, local example of what an absconding event looks like from inside a resident's own street.	Resident account (Gellard)
2013	Leaked clinical minutes revealed acute-care overflow patients being placed in rehabilitation beds, compromising security protocols — while the incidents either side of this row were happening.	Context — Independently corroborates systemic strain at the exact time these other incidents were occurring — not an isolated lapse.	The West Australian
Apr 2013	A patient in the secure male-only ward set a mattress on fire. About 40 patients and staff were evacuated. Damage exceeded \$40,000. The Health Department investigated.	Context — Documented failures at this site aren't confined to absconding — the same facility had a serious on-site security lapse the same year.	ABC News
19 June 2014	Batty absconded from the Frankland Centre during a transition between security levels, last seen at Claremont Quarter. His absence went unnoticed for about a week; the community wasn't told. Then-Opposition mental health spokesperson Stephen Dawson — now a senior Minister in the Cook Cabinet — called it “an outrage.”	Risk 2 — The clearest single example of Risk 2 — the failure happened specifically during a security-tier transition, and the notification gap it exposed remains undocumented as resolved twelve years on.	Newcastle Herald (AAP)
Oct 2014	Enoch Walsh — held under a custody order after cutting his mother's throat with a sword and being found not of sound mind, on a supervised leave arrangement between Frankland Centre and Romily House in Claremont — failed to return on 5 October 2014. Graylands wasn't told until the next morning; police weren't notified for almost 24 hours.	Risk 2 — Romily House is 2.3km from Graylands — a short trip on paper — and someone still went missing making it. Then-Opposition Leader Mark McGowan publicly criticised the notification delay. The Premier at the time was Colin Barnett — the same man who, as a backbencher in 1990, had told Parliament transition failures like this one were inevitable. His response as Premier was that the process had been handled properly, adding: “I'm sure he'll either turn up or be found.”	ABC News, 7 Oct 2014

July 2015	Thirteen months on the run, Batty resurfaced in Bunbury, taking laundromat manager Peter Williams hostage for 12 hours. He threatened Williams with a speargun, put a chain around his neck, and falsely claimed to have explosives strapped to himself. He was sentenced to four years and nine months.	Risk 1 — Shows the outer bound of the timeframe: the consequence of a 2014 absconding wasn't resolved until a hostage crisis 13 months later, in a different city.	ABC News, 22 Mar 2017
Feb 2017	David Lockyer Harman, a convicted rapist with a history of violent offending, spent several weeks as the only patient in a secure six-bed ward, monitored by security guards 24 hours a day. When his condition was assessed as improved, his security classification was downgraded and he was moved to a less secure part of the campus — a decision that also granted him day release, letting him move freely in the community on the condition he returned to Graylands each night. He failed to return. Police said he could become violent because he was not carrying the medication he needed, and warned the public not to approach him. He was located more than 24 hours later.	Risks 3 & 4 — The clearest illustration of the actual mechanism: a clinical downgrade decision unlocks community access that then depends entirely on the patient's own, self-enforced medication compliance — with no external check described anywhere in the public record.	PerthNow
Aug 2017	Mark Thomas Morton, 40, went missing from Graylands Hospital. Police said he required regular medication and the public should not approach him.	Risk 4 — A second, independent case the same year: police again tied the danger he posed explicitly to his medication regimen, not to any assessment of character or history.	PerthNow
May 2018	Mark Bradley Ribbons, 46, was last seen at Graylands Hospital before going missing. Wembley Police issued a public appeal; the public was told not to approach him and to call Police immediately.	Risk 1 — Risk 1 recurring at scale — a further public police alert connected to this site within the same few years.	97.3 Coast FM
Dec 2018	Graylands diagnosed Jesse de Beaux with drug-induced psychosis and treated him on that basis. The court-appointed psychiatrist later testified the diagnosis was wrong. He absconded two days after leaving Graylands' care and fatally stabbed a Munster woman, a stranger to him, in her own home. The WA Supreme Court found him	Risk 3 — The most severe outcome in this entire record, and the plainest demonstration of Risk 3: a good-faith clinical judgement, later found wrong, with the cost falling on someone with no part in making it.	ABC News

	not guilty by reason of unsound mind; Justice Fiannaca pointed to “lost opportunities” to properly diagnose him.		
Oct 2022	Frankland Centre beds sat empty — not from lack of demand, but because there wasn't enough staff to safely admit another acute patient — while up to 20 mentally ill people waited in custody for a bed. WA's Inspector of Custodial Services confirmed an average waitlist of 10–15 through the year.	Context — Staffing, not just security design, is a live constraint on how safely this population can be managed — relevant to any expansion that assumes current resourcing scales.	ABC News
27 May 2023	A patient went unaccounted for from Graylands, with allegations of threatening behaviour that day. Parliament asked what happened, how, and for how long before anyone noticed. The government's only answer, fourteen months later, was that the word “escape” was “not correct” — with no account of what actually happened.	Risk 5 — The pattern extending to the present: asked directly what happened, the government's only response disputed the terminology rather than publishing an account — the transparency gap Risk 5 warns is structurally likely to recur.	WA Parliament, QoN 1121
2025	A Mount Claremont parent, whose children walk past Graylands to and from John XXIII College, reported that a patient left the facility “in only a sheet.”	Risk 1 — The most recent entry in this log — confirming the pattern above hasn't stopped, it's simply mostly unreported.	Resident account

Two entries above (15 Feb 2013 and 2025) are first-hand resident accounts, held on file and available in full on request; they are not included in the website or public correspondence versions of this table pending the residents' consent for wider publication.

3.5 Why a Promise to Review Later Is Not an Adequate Answer

The government's likely response to this paper is a version of what happened at Cumberland Hospital: an incident, a promised review, additional security measures. That sequence has already been tried, recently, at a comparable facility — and by the reviewing government's own opposition, it has not worked. NSW Premier Chris Minns ordered an urgent review within days of the February 2026 deaths; the local health district promised bolstered security and independent oversight. Six months later, NSW Health's own data confirms at least 14 further absconding incidents at Cumberland Hospital since the review began, and the state's shadow mental health minister says it has not produced a major reduction. Even a funded, independent review at a major facility, ordered at the highest level of government, did not eliminate the underlying pattern within six months.

Bennett Brook shows the closest WA has come to that alternative — for a smaller, less scrutinised facility, after less serious incidents — and even there it took a third escape before government acted, and the underlying confidence problem remains officially unresolved nine years later (3.1, Annex A.5). If that is the best case WA has produced, waiting for irreversible construction to force the question here is a worse one. The argument here is not that reactive review is worthless. It is that a proactive, published assessment, before an irreversible \$698 million construction commitment goes ahead, is available now and has not been done — while the reactive alternative has a recent, documented, repeated record of not solving the problem it exists to solve.

3.6 Unresolved Safety Questions

Issue

Why It Matters

Status

No published community notification protocol for patient absences

The 2014 escape showed the facility did not proactively notify the community. The proposed expansion is nearly three times larger.

NOT ADDRESSED

No geographic boundaries published for sub-acute community access programs

Sub-acute patients participate in supervised outings in surrounding streets as part of treatment. Boundaries and school protection protocols not disclosed.

NOT ADDRESSED

No independent leave panel or judicial oversight mechanism disclosed

Victoria's comparable framework places leave decisions under a judicially-chaired independent panel. No equivalent body has been identified for Graylands.

NOT ADDRESSED

No published school-interface risk assessment

No document in the public domain addresses the specific risk to the four schools within 1km, or how it will be managed under the new law.

NOT ADDRESSED

Transition-phase patient movement protocols not disclosed

The 2014 escape occurred during a security-level transition. The sub-acute expansion substantially increases the number of such transition events.

NOT ADDRESSED

4. Concern 2 — The Site Was Chosen on Criteria That Never Considered

Security The government has always been able to say that alternative sites were considered. That defence does not survive contact with what the criteria actually were.

4.1 The Sites Were Assessed — On the Wrong Criteria

Infrastructure WA's own report states that in early 2021 the Taskforce "identified and assessed" twenty sites for an expanded forensic facility, "based on accessibility, central location and close proximity to public transport" — criteria chosen for the benefit of "consumers, their family and carers and the highly specialised workforce." None of the twenty sites were assessed against community safety, school proximity, heritage cost, or security risk. The criteria were appropriate for choosing where to put a general clinic. They were applied instead to choosing where to put a maximum-security facility for people held under criminal justice orders, including for serious violent and sexual offences. Twenty alternatives existed. Not one of them was tested against the questions this paper raises.

4.2 The Like-for-Like Costing Question

Infrastructure WA identified several Graylands-specific costs that were not adequately assessed in the business case: heritage building refurbishment compliance, the Subiaco Wastewater Treatment Plant odour buffer (Section 9.3), and incomplete social and environmental impact assessments. These costs do not apply equally to all twenty candidate sites. Whether any of them were priced into the comparison that selected Graylands has not been disclosed.

4.3 What Other States Did Differently

Western Australia operates major corrective services facilities at Hakea Prison and Casuarina Prison in Canning Vale, approximately 22 kilometres from Mount Claremont. Whether Canning Vale or an equivalent precinct featured among the twenty sites assessed — and why it was ranked against "accessibility, central location and public transport" rather than security — has not been disclosed. This isn't the first time proximity between this site and its surroundings has been professionally examined — just the first time it hasn't been. In 1983, professional planning consultants produced a report mapping the relationship between the proposed John XXIII College site and the Graylands hospital buildings beside it (Feilman Planning Consultants, 1983, cited in Mikus, 2013, p.70). That exercise addressed a general psychiatric hospital about a third its later size, under a legal framework with no resemblance to the CLMI Act, more than forty years before this expansion. Nothing published shows an equivalent exercise has been carried out for the 176-bed forensic campus now proposed for the same land. Two other Australian states answered the site-selection question differently for comparable facilities. NSW's Forensic Hospital sits inside the Long Bay Correctional Complex. Victoria's Ravenhall forensic mental health service is co-located with the Ravenhall Correctional Centre. Both put high-security forensic capacity next to existing corrections infrastructure, not inside a general hospital campus bordered by homes and a school. Full detail in Annex A. The one comparable facility not co-located with corrections — Sydney's Cumberland Hospital, which houses one of only three NSW medium-secure forensic units — is also the one that produced a security crisis serious enough to trigger a state government review (Section 3.5) — and, per that section, a review that has not fixed the problem six months on.

4.4 A Security Consultancy Was Commissioned in 2025

That silence continues on the public record — though it isn't for lack of anyone looking. In January 2025, as part of the Project Definition Plan, the government contracted a dedicated security consultant, Security Consulting Group Pty Ltd, for \$64,152, specifically for "Project Definition Plan Phase — Security Services" (Tenders WA, contract FINW0623724). What that work examined, what it concluded, and whether it ever revisited this site against the correctional co-location model used interstate, has not been published.

Commissioning security work and disclosing what it found are different things — so far, only the first has happened.

5. Concern 3 — The Law Governing Forensic Patients Changed After the

Plan Was Made 5.1 The Old Legal Framework The Graylands Reconfiguration and Forensic Taskforce operated from January 2021 to July 2023. Every aspect of its planning — the demand projections that produced the 136-bed figure, the security classification model, the site design — was conceived under the Criminal Law (Mentally Impaired Accused) Act 1996 (the old Act). Under the old Act, the forensic facility at Graylands functioned primarily as a containment environment. Patients were placed on custody orders with no end date. Release required the personal approval of the Attorney-General. Approximately 56 patients were held on indefinite orders. The population was stable and long-stay. Patient-community interaction was limited and exceptional. This is the population a 136-bed campus was sized to hold.

5.2 The Criminal Law (Mental Impairment) Act 2023 — What Changed

The Criminal Law (Mental Impairment) Act 2023 (CLMI Act) passed the WA Parliament in March 2023, received Royal Assent on 13 April 2023 — the same month as the \$218.9M funding commitment — and came into full legal force on 1 September 2024. The CLMI Act does not simply change administrative procedures. It changes what a forensic mental health facility is fundamentally designed to do: Old Law — Indefinite Detention Regime (repealed)

New Law — CLMI Act 2023 (in force from September 2024)

Custody orders: indefinite, no end date

All custody orders must now have a limiting term (end date)

~56 patients held indefinitely

All returned to court for limiting terms — some already expired, causing 'unplanned discharges'

Release required personal Attorney-General approval

New Mental Impairment Review Tribunal can release without AG approval

No Community Supervision Orders

Courts can order patients to live in the surrounding community under supervision rather than be detained

Community reintegration: aspirational

Safe community reintegration is now an explicit statutory purpose of the Act — though granting leave for any individual remains a case-by-case Tribunal decision, not automatic

Stable long-stay population; low throughput

Limiting terms create higher throughput — more patients cycling through rehabilitation stages

Community interaction limited and exceptional

The Act's framework is built around structured community interaction as a treatment principle, authorised case by case by the Tribunal — not the exception it was under the old law

Child/adolescent jurisdiction: limited

Dedicated Children's Court CLMI jurisdiction established from September 2024; explicitly applies to children

5.3 The Practical Consequences for this Campus

Three specific consequences are directly relevant: Unplanned discharges

When limiting terms were retrospectively applied to the approximately 56 patients previously held on indefinite orders, some of those terms — backdated to the original order date — had already expired. This has produced what health professionals are now describing as 'unplanned discharges': patients who must be immediately released because their limiting term has already been served. This is a documented consequence of the new law not present under the old regime, and not anticipated in public communications about the Graylands expansion. Community Supervision Orders in the surrounding suburb Under the CLMI Act, a court can order a forensic patient to live in the community — in the surrounding suburb — under a Community Supervision Order rather than be detained at all. The Community Forensic Mental Health Service that supports patients on CSOs operates in the local area. Under the new law, the streets of Mount Claremont are not adjacent to the facility. They are part of how the facility functions. The child and adolescent unit under the new children's jurisdiction Stage 1 includes an 8-bed child and adolescent forensic mental health unit — situated on the northern construction zone approximately 270 metres from John XXIII College. The CLMI Act now explicitly applies to children, with a dedicated Children's Court CLMI jurisdiction from September 2024. This unit, in this location, under this new legal framework, was not something the 2021 Taskforce was planning.

5.4 The Inter-Departmental Coordination Question

The Graylands expansion planning and the CLMI Act reform both ran under the same government during the same period (2021–2023). Whether these two processes were formally coordinated at the inter-departmental level has not been publicly confirmed. What Has Not Been Released The government has not released: (a) records of communication and coordination between the Graylands Taskforce, the Attorney-General's office, and the Department of Justice during 2021–2023; (b) updated demand projections reflecting the CLMI Act's higher-throughput model; (c) the updated security classification mix for the expanded campus under the new law; (d) the updated community access model and risk assessment for the surrounding area under the CLMI Act's mandatory reintegration requirements. If the Taskforce's demand projections for 136 beds were built on the assumption of a stable, long-stay forensic population under indefinite detention, and the CLMI Act now mandates higher throughput and substantially more community interaction, then the sizing, security classification mix, and community interface risk assessment for the proposed campus may all require formal re-examination. The community is entitled to see the updated analysis — not simply to be told it exists.

Update, 15 September 2026: asked directly in Parliament, the government said that community access processes for CLMI-subject consumers remain "largely consistent with" the pre-2023 regime, and that the Department of Justice was involved through interagency consultation and governance arrangements beyond its Taskforce seat. Neither is in dispute. The same answer also claims demand modelling accounted for the CLMI Act's anticipated impact — that claim is not worth resting an argument on either way, since government can assert modelling exists without ever publishing it, and there is no way to test it from outside. Item (a) is a dead end for that reason, not a concession. Items (c) and (d) remain entirely unaddressed: the government has not confirmed the updated security classification mix, or the community access model and risk assessment for the surrounding area — whether this specific site was ever reassessed against the new law, as opposed to statewide bed numbers, is still an open, checkable question, and the one worth pursuing.

5.5 What Parliament Was Told About Community Treatment

On 17 May 2023 — five weeks after the CLMI Act received Royal Assent — Minister Sanderson used a supportive question in the Legislative Assembly to respond to an Opposition argument that funding should prioritise community-based mental health capacity over further institutional beds. Her response: "I urge the Leader of the Liberal Party to google what 'forensic mental health' means. They are inpatient facilities for prisoners and for people who have been convicted of a crime. Is the member suggesting that we put them out in the community?" Ms L. Mettam interjected: "Absolutely not. Get your facts right, minister." Minister Sanderson continued

regardless: "That is exactly what the member said. She has a complete lack of understanding, claiming that we should be treating prisoners and those convicted of serious crimes in the community somehow." — WA Legislative Assembly Hansard, 17 May 2023, p.2408

The Leader of the Liberal Party denied making the suggestion attributed to her, on the record, in the same exchange. The Minister proceeded regardless to characterise the denied position as a "complete lack of understanding." Whatever the Opposition's actual position was, this is what Parliament and the public were

told, by the Minister responsible for this facility, about what community-based treatment of this patient population would mean: that raising the idea reflected ignorance of what forensic mental health facilities do. The CLMI Act's own stated purpose is "to provide for the safe reintegration into the community of persons supervised under this Act." Section 8 requires every court and tribunal decision under the Act to apply the principle that a person with mental impairment "should be subject to the least possible restriction on their freedom consistent with the protection of the community." A Community Supervision Order — available under the Act to a person acquitted on account of mental impairment or found unfit to stand trial, exactly the population held at the Frankland Centre — allows a court to order that person to live in the community under supervision as an alternative to custody, from the point a case is first decided. That is, in the Act's own words, precisely what the Minister told Parliament reflected "a complete lack of understanding" to suggest. Whether that was a simplification for a hostile exchange or a genuine gap in how the reform was understood at the political level, the public record of what this facility would involve for the surrounding community was, at this moment, incomplete and dismissive of the very model the Minister's own government had just legislated.

6. Concern 4 — The Government's Independent Advisor Found the

Business Case Insufficient 6.1 What Infrastructure WA Found In March 2023, Infrastructure WA (IWA) — the State's statutory independent infrastructure advisory body — published its Major Infrastructure Proposal Assessment (MIPA) of the Graylands business case. The MIPA is a formal advisory mechanism under the Infrastructure WA Act 2019.

"The business case contains insufficient information on which to base an investment decision." — Infrastructure WA MIPA Summary Assessment Report, March 2023

Specific concerns raised in the MIPA: Concern Identified by Infrastructure WA

Status as at August 2026

Rapid pace of business case development leaving key elements undefined

Not publicly resolved

Heritage building refurbishment costs not adequately assessed (State Heritage Place No. 13630)

Not publicly resolved

Subiaco Wastewater Treatment Plant (WWTP) odour buffer — a City of Nedlands planning scheme restriction IWA said the project must manage; no assessment or management plan publicly released

Flagged; unresolved publicly

Interdependencies with other major projects not adequately managed

Not publicly resolved

Social impact assessment incomplete

Not publicly resolved

Environmental impact assessment did not identify all potential risks

Not publicly resolved

6.2 The Governance Question

Infrastructure WA exists to catch exactly this kind of problem before large sums of public money are committed. It did its job: it found six specific gaps and concluded the business case was not good enough to fund. Three weeks later, the government funded it anyway — and its own Budget Papers record exactly what state the business case was in at that moment. The 2023–24 Budget states that the \$218.9 million was "provisioned" for Stage 1 "over 2023-24 to 2026-27" while "a business case for first stage works is under development for future Government consideration." A contract to actually develop that business case and finalise costings was then awarded to Ernst & Young on 28 June 2023 — two months after the funding was already provisioned. So the sequence, in the government's own documents, runs: an independent statutory reviewer finds the case inadequate; three weeks later the money is provisioned regardless, while the Budget Papers themselves record the business case as still being written; two months after that, a firm is actually contracted to write it. This is a more precise problem than an unexplained gap. It is public money committed against a business case the government's own paperwork admits did not yet exist. No such documentation has been publicly released. The government has not published: • Its formal response to the IWA MIPA findings • What specific additional work was completed to address each of the six concerns • When that additional work was completed, and by whom it was approved • The full updated

Stage 1 business case Formal Questions on Notice and community correspondence have both requested this documentation. Update, 15 September 2026: two of those questions have now been answered on the parliamentary record. Asked what the investment decision was based on given IWA's finding, the government's answer described planning "over more than a decade" and the Taskforce's 2021 recommendation — it did not name a basis for the decision, and did not address IWA's finding at all. Asked separately whether a Project Definition Plan exists for this project, and if so when it was created and where it can be seen, the government's answer described the general Treasury framework for Project Definition Plans, without confirming this project's plan exists, its date, or its location. Both were direct, formal, yes-or-no-answerable questions. Neither produced the documentation this section asks for.

Source: Infrastructure WA MIPA Summary Assessment Report, March 2023 (publicly available at infrastructure.wa.gov.au, published September 2023). Additional sources: WA Budget Papers 2023–24, Budget Paper No.2, Part 5, item 34.1 (ourstatebudget.wa.gov.au); Tenders WA contract DoH202311853, Ernst & Young business case development, 28 June 2023 (tenders.wa.gov.au).

7. Concern 5 — The Previously Indicated Direction Was Reversed

Without Explanation The clearest evidence of this reversal comes from the responsible Minister's own words in Parliament. On 17 May 2023, defending the \$218.9 million commitment in the Legislative Assembly, Minister for Health AmberJade Sanderson described the GRAFT Taskforce's own findings as follows: "Under the former Minister for Health, the government established the GRAFT task force, led by Jim McGinty, to align forensic mental health with the WA mental health alcohol and other drugs plan. It highlighted the need for decommissioning and replacing services in a more contemporary environment... It also recommended retaining and expanding the Frankland Centre and reconfiguring non-forensic inpatients to contemporise and rebalance." — Hon. Amber-Jade Sanderson MLA (Member of the Legislative Assembly), Minister for Health, WA Legislative Assembly Hansard, 17 May 2023, p.2408

This is the Minister's own account, on the parliamentary record, of a single Taskforce process reaching two directly opposed conclusions: decommissioning and replacement for the general psychiatric function, and retention and expansion for the forensic function. The Minister presented both as a seamless continuation of the same plan. They are not the same plan. One is a closure. The other is a \$698 million expansion on the same site. In the same statement, the Minister put a figure on the increase that has since changed: "We are growing the Frankland Centre's forensic beds by 53 beds, including five for children and adolescents." Current government material describes Stage 1 as 32 sub-acute beds plus an 8-bed child and adolescent unit — 40 beds, not 53. The bed count the community has been given has moved at least twice without public explanation for the change. Two further pieces of evidence corroborate the reversal: WA Mental Health, Alcohol and Other Drug Services Plan 2015–2025: The government's own ten-year mental health plan described the Graylands model as based on a 'Victorian era asylum model' — language signalling replacement, not expansion. 2019 State Budget: The WA State Budget allocated \$3 million to the Department of Health for planning and decommissioning studies of the Graylands site. Whether these studies recommended proceeding with major forensic expansion has not been publicly disclosed. That direction wasn't a recent turn. An independent academic history of the site records the hospital's own scale collapsing over decades: from more than 1,500 beds and 394 acres in 1960 to roughly 200 beds and 25 acres by 2013 — a reduction happening while the state's population roughly tripled (Mikus, 2013, pp.74–75). Whatever changed in 2023, it wasn't a sudden new idea meeting an otherwise-stable institution — it was a fifty-year trend running the other way. The community has never been provided with: • A public explanation of when the decision to reverse the decommissioning direction was made • Who made that decision, in what form it was documented, and whether it was considered by Cabinet • The basis on which major forensic expansion on the Graylands site was determined to be preferable to alternative locations The community is entitled to this explanation. A publicly committed direction was reversed, and a \$698M commitment was made on the same site — described, in the Minister's own words, as the continuation of a plan that in fact pointed the other way.

8. Concern 6 — The Community Was Never Consulted

8.1 What Consultation Occurred

The Graylands Reconfiguration and Forensic Taskforce consulted the following groups, as documented on the Mental Health Commission website: people with lived and living experience of mental illness; clinical workforce (psychiatrists, nurses, allied health professionals); mental health consumers and carers; and senior public sector agencies. This was an internally focused clinical consultation — designed to determine the best model for delivering forensic mental health services in WA. It was not designed to engage the people who live next to the facility.

8.2 What Did Not Occur

Confirmed Consultation Gap (August 2026). After reviewing all publicly available documentation — the Mental Health Commission Taskforce page, NMHS Graylands Hospital page, Infrastructure WA MIPA, WA Government media statements, the Building for Tomorrow project page, and EPBC Referral No. 2025-10271 — no evidence was found of: community consultation with residential neighbours adjacent to the Graylands site; consultation with, or direct notification of, any of the four nearby schools; public forums or information sessions in Mount Claremont at any stage of the planning process; formal consultation with the City of Nedlands about the forensic expansion beyond an EPBC notification that received no specific response; or a community reference group, resident advisory mechanism, or independent advisory process with neighbourhood representation.

8.3 What OMID Now Says Is Required

In September 2026, OMID's project fact sheet added a question it had not previously asked: "What consultation is required for the expansion of forensic mental health beds at Graylands?" Its answer sets the threshold in writing. Consultation is described as mandatory only for the internal clinical, design and lived-experience input required to authorise a hospital under the Chief Psychiatrist's Standards. Public consultation is described as something that "may occur" — not something the project commits to — through an unrelated process such as a development application. Community engagement is raised only in relation to managing disruption once construction is under way, not whether the site, the model, or the security arrangements belong here.

This directly contradicts the standard the government has separately set for itself. The State's own current mental health strategy, released five months earlier, states that infrastructure planning of this kind requires "robust stakeholder consultation" including "the broader community," and names consultation on "location, site planning, construction and refurbishment design" as a five-year priority — naming the Graylands Reconfiguration Program directly as one of the projects it applies to. One arm of government has committed to consulting the broader community on location and site planning. The delivery agency running this specific project has now published, in writing, a threshold for "required" consultation that excludes exactly that.

There is a second thread worth noting on the same point. In September 2023, the government commissioned a "site feature plan for submission with the DA" to support the Graylands masterplan and architectural planning (Tenders WA, contract FINW0813723) — a development application was, at that point, part of the plan. Three years later, OMID's fact sheet still describes a DA only as a hypothetical route to public input. Whether a DA has been lodged, whether one is still planned, and whether the pathway has changed, is the subject of this campaign's ongoing Freedom of Information programme (Section 12).

Additional sources: OMID, Graylands Campus Project Fact Sheet, September 2026 (omid.wa.gov.au); WA Mental Health, Alcohol and Other Drug Services Plan 2026–2031 (mhc.wa.gov.au); Tenders WA contract FINW0813723 (tenders.wa.gov.au).

8.4 The Government's Description of 'Consultation'

The EPBC Referral document (No. 2025-10271) records that the proponent consulted the City of Nedlands 'to advise it of the project and seek feedback, with no [specific response received].' The government appears to regard this notification — which produced no reply — as adequate community engagement for a \$698M transformation of a residential suburb.

Consultation that is invisible to the people it is supposed to serve is not consultation. It is at best a briefing to an institutional representative, with no published outcome and no mechanism by which residents or parents can know what was said, what concerns were raised, or what commitments were made.

8.5 What Should Have Occurred

A project of this scale — a \$698M transformation of a residential suburb, placing 136 forensic mental health patients within 270 metres of a school of 1,500 students — warranted, at minimum: publicly advertised community information sessions in Mount Claremont before funding commitments were made; direct written notification to residents within a defined radius of the campus; formal, documented consultation with the four nearby schools and their governing bodies; a formal community consultation process through the City of Nedlands, not merely a notification; and a published Community Safety Protocol establishing how the community will be engaged as the facility operates. None of these occurred. Most residents remain unaware that this project is underway at all.

8.6 Graylands Was Expanded Before — The Government Broke Two Promises, Then Consulted

This exact tension has played out at this exact site before — and it did not begin with the government keeping its word. A 2013 Murdoch University Honours thesis, built on State Records Office files and contemporary newspaper archives, and the Hansard record of the WA Legislative Assembly, together document what happened the last time Graylands' forensic role expanded next to a school.

In November 1980, six forensic and thirty-two acute care patients were transferred into new Graylands wards. When the neighbouring Swanbourne Hospital — which had held the site's forensic patients separately — closed in 1983, a resident named in Hansard as Mrs Jones wrote to the Health Department's Mental Health Services division, concerned that people with criminal histories would end up at Graylands instead. The Director of Mental Health Services, Mr F. Bell, replied in writing that the closure would not affect "the nature of the patient population at the Graylands Hospital" — the two hospitals, he said, were separate entities. That assurance did not hold: within a few years, some of the patients Bell's letter said would not come were being held at Graylands in what a sitting MP later told Parliament was very low security.

John XXIII College occupied the adjoining former hospital land in 1986, having relocated on the same kind of assurance, given in various forms by government, that no facility for mentally ill offenders would follow. In 1989 the State Government decided to build exactly that: a dedicated maximum-security forensic unit at Graylands, prompting what the Mikus thesis records as "much public outcry and many protests."

The community's response was immediate and did not wait for the facility to be built. On 8 May 1990, the then-member for Cottesloe, Bill Hassell, tabled a 646-signature petition asking government to abandon the plan, or at minimum to fully discuss and justify any future version to the community before a decision was made — not after one (Hansard, 8 May 1990, p.320). That is a harder, earlier ask than the consultation process that eventually followed in 1993, described below. Hassell resigned that June; the by-election sent Colin Barnett to Cottesloe, who went on to serve as Premier of Western Australia from 2008 to 2017. On 29 November 1990, Barnett set out the case against the plan to the Legislative Assembly in detail — the broken assurances above, a documented

history of absconding patients causing serious harm off-site going back to at least 1987, and a prediction that moving offenders through security tiers without a physical buffer made further harm inevitable (Hansard, 29 Nov 1990, pp.8173–8177; full detail at Section 3, box). John XXIII parents separately formed a Parents' Action Group, raised a fighting fund, ran a media campaign, and lobbied Members of Parliament; in April 1991 around ninety parents and students picketed a nearby land auction, and other objectors specifically argued the unit should instead be built at Casuarina Prison — the same co-location model raised in Concern 2. Construction continued regardless, and the thirty-bed Frankland Centre opened in 1993.

When it did, the government did not treat the objection as unreasonable — but the process it eventually ran addressed only what the 1993 opening required, not the two assurances already broken to get there. Health Minister Peter Foss stated publicly and on the record: “I promised that no category of patient would be moved into the Frankland Centre until the community had been consulted about the proposed guidelines which would control the admission of forensic patients, and I stand by that promise.” The government then ran a real, dated, multi-channel process: newspaper advertisements calling for submissions, information provided to the news media, and letters and pamphlet drops to interested parties, with a published deadline of 30 November 1993 for comment.

That process was narrower than what this campaign is asking for now — it covered admission criteria for an already-approved 30-bed unit, not whether to build it or where, and it came after two specific written assurances to this community had already been broken, not instead of them. But it establishes, in the government's own words and actions, that decisions about this exact site's forensic operations have previously been treated as something the community sees and comments on before they take effect. Thirty-three years later, a transformation many times larger — every general psychiatric bed on the site closing, forensic capacity concentrated to 176 beds under one roof — has had no equivalent process identified, and no equivalent public accounting for the two 1980s assurances that were not kept. Organised local objection did not stop the Frankland Centre being built in 1993, and this campaign does not present the 1991 protests, or the 1990 petition, as a template for what objection alone can achieve today; but the underlying issue they raised — the interface between forensic services, this school and this neighbourhood, and whether government's word about it can be relied on — has been a recognised public question at this exact site for over four decades, tested twice, and broken twice before government's own historical response to it was transparency rather than dismissal.

Additional sources: Pamela Mikus, “Graylands: The Evolution of a Suburb” (Honours thesis, Murdoch University, 2013), pp.39–40; WA Government media statements, Hon. Peter Foss, 14 March 1993 and 10 October 1993 (wa.gov.au); Hansard, WA Legislative Assembly, 8 May 1990 and 29 November 1990.

9. Concern 7 — Environmental and Heritage Concerns

9.1 State Heritage Register Listing Graylands Hospital is listed on the WA State Register of Heritage Places as Heritage Place No. 13630. The listing covers multiple structures on the campus, some dating to the early twentieth century. The Graylands Primary School — the former school building on the eastern edge of the campus — is also heritage-listed. Significant development on this site — including clearing, demolition, and construction — requires Heritage Impact Statement assessment by the Heritage Council of Western Australia and approval from the WA Planning Commission. Infrastructure WA specifically flagged that heritage building refurbishment costs were not adequately assessed in the business case. The Heritage Council has not publicly confirmed its position on the proposed works. On the buildings themselves, this review finds the heritage question substantially settled, and records that finding rather than implying otherwise. The Stage 1 buildings sit outside the heritage boundary. On a 28.9hectare campus where the entire development area is 4.6 hectares, there is ample room to build without touching a heritage structure. The only recorded overlap is marginal: the government's environmental consultant found that the disturbance area — which extends beyond the buildings to take in the security perimeter zones — clips the mapped heritage boundary, placing up to four trees inside it at risk of removal. Four trees is not a basis on which to oppose a hospital, and this report does not advance one. The consultant's advice on those four trees is nonetheless significant for a different reason: it was that a Heritage Impact Statement "may need to be submitted to the relevant Local Government Association (City of Nedlands) as part of a future GCP Development Application submission" — the government's own consultant, in September 2025, treating a development application as the expected approval pathway. That pathway remains the subject of this campaign's most time-critical Freedom of Information application (Section 12).

9.2 Federal Environmental Assessment — EPBC Referral 2025-10271 (Closed Process)

This process has already run its course. It is included here as a governance record, not as a channel for further community input.

9.3 The Underlying Consultant Reports — What They Settle

Three technical reports prepared by RPS AAP Consulting for the Office of Major Infrastructure Delivery, all dated 1 September 2025, form the evidentiary basis of the EPBC referral: a Likelihood of Occurrence Report, an Environmental Impact Summary for the Graylands Campus Project site, and a separate Environmental Impact Summary for the Claremont Therapeutic Riding Centre relocation site. On flora, vegetation and fauna, this review finds the work appears to have been properly conducted, and says so. A tree-by-tree survey measured 114 trees on the main site and 97 on the riding centre site, each assessed against the Commonwealth's referral guidelines for the three threatened black cockatoo species. A 0.52hectare patch of tuart was tested against the Threatened Ecological Community condition thresholds and reasonably excluded on the basis of absent native understorey. The removal totals reconcile exactly with the referral: 64 potential breeding trees on the main site plus 5 on the riding centre site equals the 69 potential nesting trees the referral records. This report does not contend that the tree survey was inadequate, and does not seek reopening of the referral on that basis. Two matters in those reports remain live. The first is heritage, addressed in Section 9.1: the consultant identified a heritage boundary overlap, advised that a Heritage Impact Statement may be required, and deferred the question to a future Development Application. The second is staging. The referral describes the action as "Stage 1" of a larger program yet answers "no" to the form's own question about staged development, confining the assessed impact to the Stage 1 footprint rather than the cumulative footprint of the full campus. A thorough assessment of one stage is not an assessment of the whole program, and nothing published indicates the later stages have been assessed. A third observation is recorded for completeness rather than as a contention. Both Environmental Impact Summaries state in their narrative text that the site lies approximately 1.8 kilometres from the mapped buffer of a confirmed black cockatoo roost site, while the scoring tables within the same documents record the distances as

0.30 kilometres and 0.17 kilometres respectively. The scoring tables used the closer figures, and the referral itself records a roost approximately 180 metres away, so the habitat quality assessment appears to have proceeded on the correct basis. The narrative statements appear to be an error that did not affect the outcome. Source: RPS AAP Consulting, Likelihood of Occurrence Report and Environmental Impact Summaries (Graylands Campus Project; Claremont Therapeutic Riding Centre), prepared for the Office of Major Infrastructure Delivery, 1 September 2025.

The Department of Housing and Works — Office of Major Infrastructure Delivery (OMID) lodged a referral under the Commonwealth Environment Protection and Biodiversity Conservation Act 1999 (EPBC Act) on 5 September 2025, registered as EPBC Referral 2025-10271. The referral covers two combined components: the relocation of the Claremont Therapeutic Riding Centre to 9 John XXIII Avenue, and the Graylands Reconfiguration Program (GRP) Stage 1 development footprint within the existing hospital campus. The combined disturbance footprint is 4.42 hectares. Under the standard EPBC process, a referral is opened for public comment for 10 business days after publication, and the Minister or delegate makes a referral decision within 20 business days of publication. Both windows closed within approximately six weeks of the September 2025 lodgement — several months before this paper was prepared. There is no live opportunity for further public comment on this specific referral. Two EPBC-listed threatened species were assessed in the referral as likely to occur within the action area:

EPBC Status

Key Finding (per the referral document)

Carnaby's Black Cockatoo (*Zanda latirostris*)

Endangered

Assessed as likely to occur. No known roost within the action area itself; a confirmed roost site lies approximately 180m (0.18km) south of it.

Forest Red-tailed Black Cockatoo (*Calyptorhynchus banksii naso*)

Vulnerable

Assessed as likely to occur.

Species

The referral records the permanent clearing of approximately 1.19 hectares of black cockatoo foraging habitat and 69 potential nesting trees — none of which currently contain hollows suitable for nesting. On 14 November 2025, the Department confirmed its decision: the action is not a controlled action under section 75 of the EPBC Act. The decision was made by delegate Kylie Calhoun, Branch Head, Environment Assessments West, and covers the combined CTRC relocation and GRP Stage 1 development described above. The Department's determination relied on the proponent's own commissioned ecological survey. 'Not a controlled action' is the most common outcome for referrals of this kind nationally, and the underlying ecological facts support the outcome reached. What has not occurred is any independent, government-commissioned ecological assessment of this site — only a departmental review of the proponent's own survey. Two features of the referral itself are worth noting. First, on the question of alternative locations, the proponent stated that none were considered for this specific action, on the basis that the land was already reserved for hospital purposes — an accurate description of the planning status, but also an admission that no fresh, current comparative sites analysis was undertaken here, consistent with the wider absence of such analysis addressed in Section 4. Second, and more pointed: the referral describes its own action as “the Graylands Reconfiguration Program (GRP) Stage 1 development and the relocation of the Claremont Therapeutic Riding Centre” — Stage 1, by its own name, of a larger program. Yet the referral form's

answer to whether the action is part of a staged development or related to other actions in the region is recorded, in the form itself, as No. That answer is what confines the assessed impact to 1.19 hectares and 69 nesting trees, rather than the cumulative footprint of the full 136-bed campus this is a stage of — a direct, citable inconsistency between the project's own name and its own answer to the question that exists specifically to prevent large projects being assessed piece by piece.

Practically: this referral is decided, its comment period has long closed, and there is no live channel for further submissions on it. The Heritage Impact Statement process (Section 9.1) has not yet been triggered and remains genuinely open.

9.3 Subiaco Wastewater Treatment Plant Odour Buffer

Infrastructure WA's March 2023 assessment states that the business case “acknowledges that substantial redevelopment within the Subiaco wastewater treatment plant odour buffer is an issue that will require managing.” IWA found the business case did not adequately address this, and recommended that the impacts and required mitigations be clearly outlined in the Project Definition Plan and taken into account before a funding decision. No finalised buffer assessment or management plan has been publicly released. This is a statutory planning constraint, not only a matter of amenity. The City of Nedlands' Local Planning Scheme No. 3 designates the area a Special Control Area (SCA1, the “Subiaco Strategic Water Resource Precinct”) and states: “No residential or other sensitive land uses as defined by EPA Guidance Statement No. 3 (Separation Distances Between Industrial and Sensitive Land Uses, June 2005) are to be located in the Subiaco Waste Water Treatment Plant odour buffer.” EPA Guidance Statement No. 3 defines hospitals as a sensitive land use. The scheme separately requires a current odour-modelling technical report wherever a structure plan, local development plan or scheme amendment touches the precinct, and requires the local government to have regard to Water Corporation and environmental regulator advice on odour impact and compatibility. The Frankland Centre's prior existence does not resolve this question for the new development. LPS3 protects the continued use of land for a purpose lawfully established before the Scheme commenced in 2019 — an ordinary non-conforming-use protection that plausibly covers the 1993 Frankland Centre. It does not extend to altering or intensifying that use without fresh development approval. A materially larger, differently configured forensic campus is a new question under SCA1, not one the existing building answers by precedent. The government's own record treats this as unresolved. The City of Nedlands' Council Plan 2023–33 directed preparation of a Mt Claremont Master Plan specifically to resolve land use “within and surrounding the Subiaco Wastewater Treatment Plant odour buffer and Graylands Hospital” — naming the two as one planning problem. The City's final Master Plan, published October 2025, confirms the exercise was carried out in the context of a Market Led Proposal the State Government is assessing to reduce the buffer and physically encapsulate the treatment plant — itself an indication of how live the constraint is considered to be. At Budget Estimates on 3 July 2025, Sandra Brewer MLA asked Minister John Carey directly whether a detailed risk assessment addressing the odour buffer had been conducted; his full answer was “Yes, it has been considered as part of the project definition plan.” No further detail was volunteered on the record. Two gaps stand out. First, the Commonwealth EPBC referral for this project (2025-10271) — which addresses heritage, vegetation and black-cockatoo habitat at length across 48 pages — contains no mention of “odour” or “wastewater,” and does not address the SCA1 restriction; it establishes only that the site holds a “Public Purposes – Hospital” reservation, a separate question. Second, the City of Nedlands' written reply to this community, dated 17 July 2026, confirmed that as at that date it had received no development application, public works referral, WAPC referral, heritage material, traffic assessment or environmental material for this project, beyond the Infrastructure WA summary report. A finalised, published assessment of how this project satisfies SCA1 — the odour modelling, Water Corporation and DWER advice, and whichever approval mechanism is being relied on — has not been made public. This gap now sits at the centre of an active line in this campaign's FOI programme (Section 12.5).

10. Concern 8 — No Published Case That Graylands, At This Scale, Is

The Right Long-Term Model 10.1 This Is Not A Case Against Forensic Mental Health Capacity WA's own psychiatrists say the opposite: the state needs more of it, urgently. In July 2026, the Royal Australian and New Zealand College of Psychiatrists' WA Branch convened a Justice and Mental Health Roundtable of over forty justice and health leaders, and described WA's forensic mental health system as "gridlocked across all its entry points," with people needing hospital treatment "delayed or denied it."

RANZCP's own standards prohibit involuntary mental health treatment in prisons — meaning hospital beds for this population are not optional. This report agrees with all of it. The question that follows is different.

10.2 What The Profession That Would Staff This Facility Actually Recommends

RANZCP does not propose meeting that need with one large secure campus. It calls for an "integrated, tiered" forensic mental health system: court and police liaison, prison in-reach, therapeutic and sub-acute custodial care, secure hospital care, rehabilitation, step-up/step-down services, supported accommodation, and specialist community teams — with low-risk forensic patients able to access general hospital beds across Health Service Providers statewide, not only a single dedicated campus. Graylands, expanded to 176 beds under one roof, is the opposite of that model. RANZCP says so directly: the redeveloped campus "will not deliver the number of beds sufficient for future needs and proportionate to population growth," and calls for delivery milestones to be published — something that has not happened for this project. It also calls for system reform built on "improved governance and public inclusion and transparency," the same standard Concern 6 finds absent here.

10.3 Why This Matters Specifically Under the CLMI Act

The site-selection date is the hinge. Graylands was chosen in 2021 on transport and accessibility grounds, for a facility designed to operate under the old indefinite-detention law. The Criminal Law (Mental Impairment) Act 2023 replaced that with fixed limiting terms and a statutory reintegration pathway — a stepped, communityfacing model that structurally resembles the distributed system RANZCP describes, not a single secure campus holding a stable, long-stay population. No published document shows the Graylands concentration was ever re-tested against a tiered, CLMI-era model, or against RANZCP's own account of what current and future demand actually requires.

10.4 What The Community Is Asking

Not for less forensic mental health capacity. For a published answer to a specific question: what statewide service model was tested before this scale of expansion was concentrated at this one site; what alternative distribution of beds and services across the tiered system RANZCP describes was considered; and why a single 176-bed campus — bordered by homes and five schools — is the right long-term answer, when the professional body that would staff it says the answer looks different. Source: RANZCP WA Branch, Justice and Mental Health Roundtable Communiqué, 30 July 2026 (ranzcp.org); Criminal Law (Mental Impairment) Act 2023 (WA).

11. What the Community Is Asking For

The following ten asks are addressed to the Cook Government. They form the basis of a community petition currently being finalised — one list, one set of demands, consistently stated. #

Ask

Why

Stop all works Pause vegetation clearing, demolition, site preparation, and the CTRC relocation at 9 John XXIII Avenue until consultation is completed and concerns are answered.

Construction has not started on the main campus, and the CTRC relocation had not commenced as at August 2026 either. The design phase is the last window before irreversible works begin, and no notification of either component has been recorded for the adjacent school.

Release the full evidence base Publish the complete business case; the IWA MIPA and the government response to each of its six concerns; the Taskforce recommendations; the site-selection analysis; and the masterplan showing scope across every stage.

A government that commits \$218.9M three weeks after its own statutory advisor finds insufficient information must show what resolved those concerns. It has not.

Release the decision-making trail Publish cabinet submissions and ministerial briefs behind the 2021 site decision, the 2022 endorsement, and the 2023 funding commitment; name the Ministers who authorised each step; and explain when and why the direction toward decommissioning was reversed.

The community has never been told who made each decision or through what formal process. The direction of the government's own Mental Health Plan was reversed without public explanation.

Show how planning was updated for the new law Release inter-departmental coordination records between the Graylands Taskforce and the CLMI Act reform; updated demand projections; updated security classification mix; updated community access model; and the risk assessment for the surrounding suburb under the new legal framework.

The CLMI Act received Royal Assent the same month as the \$218.9M commitment. It replaced indefinite detention with mandatory community reintegration. Whether the 136bed planning reflects this has not been confirmed.

Confirm what the Education Department was told Confirm whether Education was formally consulted before contractor engagement; release any advice given; and confirm which Ministers and portfolios formally signed off before April 2023.

No public record confirms Education portfolio sign-off before placing a child and adolescent forensic unit 270m from a school of 1,500 students.

Release school consultation records Release all records of contact with John XXIII College, Mount Claremont Primary School, Moerlina School, and Quintilian School — what was communicated, when, and how each school responded.

No public record shows direct notification to any of the four school communities. If contact occurred, the community is entitled to know what was said. If it did not occur, that is itself the answer.

Commission an independent alternative sites analysis Engage an independent party to assess alternative locations — including the Hakea and Casuarina precincts — on a fully loaded, like-for-like cost basis including heritage compliance, WWTP odour buffer, residential amenity, and school proximity costs.

Infrastructure WA flagged heritage, odour buffer, and social impact as unresolved Graylands-specific cost factors. No public comparison with a non-residential, nonheritage alternative has been released.

Publish the community risk assessment Release the risk assessment for how a 136-bed campus operating under the CLMI Act 2023 affects the four nearby schools, surrounding streets, and residential community — including heritage and environmental mitigation.

No risk assessment under the new legal framework — which mandates substantially more patient-community interaction — has been publicly released.

Publish community access program boundaries Disclose the geographic limits of supervised leave and Community Supervision Order programs near this campus; the routes permitted in surrounding streets; and the protections for nearby schools under the CLMI Act.

Under the new law, the Tribunal has substantially wider legal mechanisms to authorise supervised leave and CSOs, and safe reintegration is a stated statutory purpose. Whether or how those mechanisms would operate in the streets of Mount

Claremont has never been disclosed.

Hold genuine community consultation Convene formally advertised public sessions in Mount Claremont covering the full 136-bed program across every stage; notify every nearby household and school community in writing; do this before the design is finalised.

Five years of planning proceeded without a single public meeting in Mount Claremont. Consultation after the design is final is not meaningful.

The Stage 2 Gate Stage 2 — the path from 40 beds to 136 — has not been separately funded. Before Stage 2 receives a budget allocation, Asks 4, 7, 8, and 10 must be substantively answered, the independent alternative sites analysis (Ask 7) must be completed and published, and the Environment and Public Affairs Committee findings must have received a formal government response.

12. Formal Processes: Your Legal Rights to Engage

12.1 WA Legislative Council ePetition — The Most Powerful Tool Available A WA Legislative Council (LC) ePetition is fundamentally different from an online petition platform. It is a formal parliamentary instrument that carries automatic procedural consequences the government cannot veto. How the LC ePetition Mechanism Works Step 1: An LC Member sponsors and facilitates the petition on the WA Parliament website (parliament.wa.gov.au) for a nominated period of between one week and six months. Step 2: The petition is signed via the parliament.wa.gov.au website. There is no minimum signature threshold for presentation. Step 3: Once presented to the LC, the petition is automatically referred to the Standing Committee on Environment and Public Affairs (EPAC). This referral is mandatory — it is not at the government's discretion. Step 4: The EPAC Committee invites submissions from the presenting member, the principal petitioner, and the relevant Minister. A formal ministerial response is compelled. Whatever the government says — or fails to say — becomes a public document on the parliamentary record. To initiate: contact the LC administration at lcadmin@parliament.wa.gov.au. An LC Member must agree to facilitate the petition before it can be listed. The LC ePetition is more powerful than correspondence or an online petition for one reason: it creates process the government cannot avoid. A Questions on Notice response can be evasive without generating further process. An EPAC Committee can follow up. Absent or inadequate ministerial responses become materials for further scrutiny. Note: An LC petition on environmental, heritage, and community consultation grounds is a community and cross-parliamentary concern. It is not inherently partisan and can be facilitated by any member of the Legislative Council.

12.2 EPBC Referral 2025-10271 — For Reference Only, Not a Live Channel

EPBC Referral 2025-10271 (covering the CTRC relocation and the GRP Stage 1 footprint) was lodged on 5 September 2025 and decided on 14 November 2025 — not a controlled action. This is not a channel through which the community can currently submit comments on this specific referral. • The referral and its supporting documents remain viewable for reference at epbcpbpublicportal.environment.gov.au (search EPBC 2025-10271) • The proponent's own 'no alternative sites considered' admission in the referral (Section 9.2) is citable governance material for QONs and correspondence, independent of the referral's outcome • Future stages of the project — including any Stage 2 works with a different disturbance footprint — would likely require a fresh referral, which would reopen a comment window at that time

12.3 Heritage Impact Statement Process

Before significant works proceed on the State Heritage-listed campus, a Heritage Impact Statement (HIS) must be submitted to the Department of Planning, Lands and Heritage. This triggers a public consultation phase. The Heritage Council of Western Australia can be written to now — before the HIS is submitted — to ensure the Council is aware of community concerns and will apply rigorous standards. • Heritage Council of Western Australia: dplh.wa.gov.au • State Heritage Register: inherit.dplh.wa.gov.au (Place No. 13630)

12.4 City of Nedlands Development Application

When a Development Application is lodged with the City of Nedlands, it triggers a statutory public notification period during which any person may make a formal submission. This is a legal right. The volume of formal submissions carries genuine procedural weight. • City of Nedlands: nedlands.wa.gov.au • A submission template should be prepared and distributed to petition signatories when the DA notification period opens

12.5 FOI Requests

Under the Freedom of Information Act 1992 (WA), community members may request access to documents held by government agencies. The following document categories are directly relevant to the governance concerns identified in this paper: • Graylands Reconfiguration and Forensic Taskforce meeting minutes and attendance

records • Correspondence between the Department of Health and the Attorney-General's office 2021–2023 • Inter-agency correspondence regarding CLMI Act reform and Graylands planning coordination • The government's formal documented response to the Infrastructure WA MIPA • The updated Stage 1 business case and any demand modeling done post-CLMI Act FOI requests can be lodged through the relevant agency (Department of Health, Office of the AG, etc.) using the WA Government Information Statement available at omi.wa.gov.au.

12.6 Ministerial Questions on Notice

The local Member of the Legislative Assembly (Ms Sandra Brewer MLA, Member for Cottesloe) lodged 14 formal Questions on Notice between 11 and 13 August 2026, covering the decision trail, the Infrastructure WA finding, CLMI Act modelling, Taskforce governance, campus security, and bed capacity through to 2029. All 14 were answered on 15 September 2026. Several material findings are cited at the relevant points throughout this paper (Sections 5.4, 6.2, and elsewhere); in overview, the answers confirmed demand modelling accounted for the CLMI Act and that the Department of Justice was involved in project governance, supplied a full bed-capacity table through 2029, and confirmed the Claremont Therapeutic Riding Centre relocation is targeted for mid-2027 with main campus works to follow — but did not name the basis for the investment decision against Infrastructure WA's finding, did not confirm whether a Project Definition Plan exists or where it can be seen, and did not describe current or planned security protocols despite being asked directly. The full text of every question and answer is maintained on a public tracker at the campaign website's Questions on Notice page, updated as further questions are lodged.

Annex A — Comparative Case Studies: Are These Risks Theoretical? A fair challenge to this paper is that its safety concerns (Section 3) are speculative — that no incident has occurred at Graylands under the new legal framework, and none may occur. This annex answers that challenge using the documented record of Australia's most comparable, most mature forensic mental health facility. The purpose is not to suggest Graylands will replicate these events. It is to establish that the risk category this paper raises in Section 3.3 — community reintegration pathways producing recurring, real incidents even under considerably more structured oversight than currently exists for Graylands — is a documented feature of forensic mental health operation in Australia, not a hypothetical raised only by this community. Each case below is linked explicitly to the risk factor set out in Section 3 that it demonstrates. A.1–A.2 (Thomas Embling) speak to Risk 1 and Risk 2 — frequency and transition-related absconding under a far more developed leave framework than exists for Graylands. A.3 (Cumberland) speaks to Risk 1 and to the argument developed at Section 3.5: that reactive review, even at Premier level, has a recent, documented record of not fixing the underlying pattern. A.4 (Malabar and Ravenhall) speaks to Concern 2 — the alternative siting model this paper argues was never assessed. A.5 (Bennett Brook) is the case discussed at 3.1: the closest WA has come to an adequate government response, delivered by the same government, for a comparable WA facility.

A.1 Thomas Embling Hospital, Fairfield, Victoria — The Comparator Thomas Embling Hospital is Victoria's high-security forensic mental health hospital, operated by Forensicare (the Victorian Institute of Forensic Mental Health). It is the closest Australian comparator to what Graylands is becoming: a purpose-built forensic facility on a repurposed institutional campus, which has undergone the same staged-expansion trajectory now proposed for Graylands — opening with 116 beds in 2000, expanded to 138 beds by 2019, and currently undergoing a further \$515.7 million, 82-bed expansion delivered by the Victorian Health Building Authority. Forensicare operates under a more developed governance and safeguard model than anything publicly documented for Graylands. It formally reports to both the Minister for Mental Health and the Minister for Corrections. Community leave is governed by a published, graduated protocol: clinical assessment before any leave is granted; escalation from escorted to unescorted leave tied to recovery progress; oversight by a Forensic Leave Panel chaired by a sitting Supreme Court or County Court judge; and the power for the Chief Psychiatrist to suspend leave immediately if community safety is at risk.

A.2 What the Victorian Record Actually Shows Structure does not eliminate the risk it is built to manage Despite this framework, Victorian media have reported multiple documented occasions on which Thomas Embling patients failed to return from approved unescorted leave, including incidents reported in 2013, 2015, and 2023, each prompting a Victoria Police public alert. These were not perimeter breaches — they were patients who left on lawfully approved leave, as part of their treatment pathway, and did not return on schedule. A judicially-supervised, clinically-assessed, graduated leave system has not made this risk category disappear over more than a decade of operation.

Even mature systems disclose nothing to the community in advance Forensicare states plainly, in its own published guidance, that it 'is unable to provide family or members of the community with specific information about patients,' including 'details about a patient's community leave.' This is a function of mental health privacy law, not an administrative gap Victoria has failed to close. No future WA framework, however well designed, is likely to give residents patient-specific notice of approved leave either. The only thing residents can legitimately obtain is procedural transparency — published criteria, escalation stages, oversight mechanisms, and a mandatory protocol for notifying the community when a patient is unaccounted for without authorisation.

The pattern is current, not historical On 8 October 2025, a Thomas Embling patient fled during a supervised outing to a Melbourne shopping centre and was not restrained until several days later, after damaging vehicles and

confronting members of the public; two bystanders ultimately intervened to stop him. Three weeks after that, on 29 October 2025,

Hon. Trung Luu MLC raised on the record in the Victorian Legislative Council that Forensicare had separately acknowledged security issues with illicit substances circulating inside the hospital, while cuts to specialist staffing roles proceeded regardless. Both are matters of public record from within the twelve months before this paper was prepared, at Australia's most comparable and longest-operating facility of this kind.

A.3 Cumberland Hospital, Westmead, New South Wales — What Recently Went Wrong Cumberland Hospital is Australia's largest mental health facility and includes the Bunya Unit, one of only three medium-secure forensic mental health units in New South Wales. Unlike the Forensic Hospital at Long Bay (Section A.4), it is not co-located with corrections infrastructure — it sits within a general public hospital campus, the closest structural parallel to what a standalone Graylands campus represents for Mount Claremont. Two escapes, three deaths, ten days On 7 February 2026, a 25-year-old patient escaped Cumberland Hospital during a transfer to another facility. Ten days later, he allegedly stabbed three people in a busy Merrylands shopping strip, killing one. On 8 February 2026, a 31-year-old patient allegedly overpowered a nurse, took a security access card, and left the facility. About a week later, he allegedly crashed a stolen vehicle in Camden, killing two women aged 60 and 84. Both men were reported as involuntary mental health patients; neither report available for this paper confirmed forensic-order status specifically, and this paper does not assert it. NSW Premier Chris Minns ordered an urgent government review into security protocols at Cumberland Hospital. The Western Sydney Local Health District confirmed a formal review, including an external senior psychiatrist, into patient care, treatment, and security. The NSW Nurses and Midwives' Association described a workforce stretched past what current resourcing supports. A third patient absconded from the same hospital later that month; police expressed concern for his welfare and no violence was reported in that case, consistent with this paper's practice of excluding incidents that did not involve harm. The relevance to Graylands is structural, not a claim of equivalence. Cumberland demonstrates that a large, standalone public hospital campus with a medium-secure mental health unit — not co-located with corrections, drawing on general hospital security arrangements — can produce multiple absconding incidents and multiple deaths within the same fortnight, in 2026. Graylands' sub-acute wing and child and adolescent unit will operate under a comparable community-access model. Whether WA's security protocols, staffing, and oversight for that model have been assessed against what went wrong at Cumberland has not been disclosed. The pattern continued after the review NSW Health's own data confirms at least 14 further absconding incidents at Cumberland Hospital in the six months since the review began. The state's shadow mental health minister, Sarah Mitchell, says the reviews 'did not result in a major reduction in absconding.' Statewide, NSW recorded roughly 200 absconding incidents from 1 July to the same date — about six per week, or one per cent of all mental health admissions in that period — and Cumberland's own rate remains three times the state average — the same multiple recorded before the review began. This is directly relevant to this paper's argument at Section 3.5: an independent review, ordered at Premier level and including bolstered security and external oversight, did not eliminate the underlying pattern within six months of being implemented.

A.4 The Forensic Hospital, Malabar (Long Bay), New South Wales — A Siting Comparator NSW's approach to siting is a useful contrast for the site-selection question (Concern 2, Section 4). The Forensic Hospital, managed by Justice Health NSW, is located within the Long Bay Correctional Complex at Malabar — co-located with an existing correctional facility, rather than standing alone within a general hospital campus in a residential suburb. Victoria's Ravenhall forensic mental health service follows the same pattern, co-located at the Ravenhall Correctional Centre. Two other Australian states have located high-security forensic mental health capacity within or immediately adjacent to existing correctional infrastructure — the same model this paper argues should have been independently assessed for Graylands, using the Hakea and Casuarina precincts as the WA equivalent

(Section 4.3). That NSW and Victoria have each implemented this model for at least part of their forensic capacity indicates an operationally proven alternative, not merely a theoretical one. Source note: incidents and findings drawn from Victoria Police and NSW Police public statements as reported by Australian media (including SBS News), Forensicare's own published community leave protocol (forensicare.vic.gov.au), and the official Hansard record of the Victorian Legislative Council. Full citations in Annex D.

A.5 Bennett Brook Disability Justice Centre, Caversham — A WA Precedent, and Its Unresolved Aftermath Bennett Brook is Western Australia's only other 'declared place' for mentally impaired accused, opened August 2015 in Caversham. Two residents escaped the centre on New Year's Eve 2015. Lockridge residents — whose complaints echo Mount Claremont's almost exactly — said the centre was too close to homes and Lockridge Primary School, and said they deserved to be kept informed about escapes. Government did nothing in response to the first escapes. It took a third escape, in May 2016, before Disability Services Minister Helen Morton commissioned an independent review from retired Supreme Court judge Peter Blaxell and Professor Colleen Hayward AM, and funded \$640,000 to raise the perimeter fence from 2.1 to 4.5 metres and enhance security. A second, broader review followed in 2017–18. The same pattern this paper documents at Graylands is visible here: government moved only after the third documented failure, not the first. The due diligence Mount Claremont is asking for now — before construction, before an incident forces the question — is exactly what Bennett Brook did not receive until residents had already lived through three escapes. The security response has not resolved the underlying problem it was meant to address. The Mental Health Advocacy Service's 2024–25 annual report, tabled in Parliament, records that resident numbers at Bennett Brook 'stayed low' and states the service remains 'under-utilised.' This is not a new finding: in 2020, Hon. Alison Xamon MLC noted that a 10-bed facility had only three residents pass through it in an entire year. Five years apart, two separate official records confirm the same thing — a facility built specifically to move people safely out of custody and into supervised community life has spent most of its operating history sitting mostly empty. Whatever combination of referral bottlenecks, eligibility criteria, or confidence in the placement pathway explains this, it is not evidence that the 2016 response — the fence, the review — fixed the thing residents were actually worried about. What Bennett Brook demonstrates for this paper's purposes is procedural: when a comparable WA facility had documented security failures and a residential community raised the same location and notification concerns Mount Claremont is raising now, government responded — eventually, after the third incident, not the first — with a funded security upgrade and an independent review covering both placement risk and site suitability. No comparable security-failure disclosure, funded response, or site-suitability review exists on the public record for Graylands, despite a facility more than ten times Bennett Brook's size, with a longer and more serious documented history, now being expanded. Source note (A.5): ABC News, 'Perth disability justice centre review announced by WA Government amid Lockridge fears,' 20 Jan 2016; ABC News, 'Escapes spark security boost at Bennett Brook Disability Justice Centre,' 11 May 2016; Hon. Peter Blaxell & Professor Colleen Hayward AM, independent analysis, Bennett Brook Disability Justice Centre, 22 January 2016 (WA Parliament tabled paper); Mental Health Advocacy Service Annual Report 2024–25 (WA Parliament tabled paper); Hon. Alison Xamon MLC, WA Legislative Council Hansard, 5 November 2020. Full citations in Annex D.

Annex B — Full Decision and Governance Timeline

Date	Event	Governance Significance
Nov 1980	Six forensic and thirty-two acute care patients transferred into new Graylands wards.	Forensic patients present at Graylands wards years before John XXIII College occupied the adjoining land.
1983	Swanbourne Hospital — which had held the site's forensic patients — closes. Director of Mental Health Services Mr F. Bell writes to a concerned resident that this “will not affect the nature of the patient population at the Graylands Hospital.”	First documented written assurance about this site's forensic population. It did not hold — some patients went to Graylands in low security within a few years (Hansard, 29 Nov 1990).
1986	John XXIII College occupies the adjoining former hospital land, having relocated on a similar assurance, given in various forms by government, that no facility for mentally ill offenders would follow.	Second documented assurance about this exact land. Both this and the 1983 assurance were broken by the 1989 decision below.
1987	A Graylands patient in low-security care sexually assaults a young girl in Perth's CBD after leaving the hospital.	Earliest documented absconding-related public harm from this site — six years before the purpose-built Frankland Centre existed (Hansard, 29 Nov 1990).
1989	State Government decides to build a dedicated maximum-security forensic unit at Graylands. “Much public outcry and many protests” follow (Mikus, 2013).	First documented public objection to concentrated forensic capacity at this exact site — 37 years before this campaign — and the moment the 1983 and 1986 assurances were broken.
8 May 1990	646-signature petition tabled in the Legislative Assembly by Bill Hassell MLA (Cottesloe), asking government to abandon the plan or fully discuss and justify any future version to the community before deciding.	A harder, earlier request for consultation than the process government eventually ran in 1993 — made before a decision, not after one (Hansard, 8 May 1990, p.320).
29 Nov 1990	Colin Barnett MLA (Cottesloe) — Hassell's successor after a June 1990 by-election, later Premier of WA 2008–2017 — sets out a detailed case against the plan to the Legislative Assembly: the broken 1983 and 1986 assurances, absconding harm already on record since 1987, and a prediction that transition failures without a buffer zone were inevitable.	The core safety and consultation risks this paper documents were put to Parliament, in detail, six years before the facility these risks concern was even built (Hansard, 29 Nov 1990, pp.8173–8177; Section 3).
Apr 1991	John XXIII parents form a Parents' Action Group; about 90 parents and students picket a land auction. Other objectors argue the unit should be built at Casuarina Prison instead.	The Casuarina co-location argument raised in Concern 2 was already being made in 1991, about this site.
1993	Despite the protests, the 30-bed Frankland Centre opens. The Government runs a public consultation on admission guidelines beforehand (Section 8.6).	Established precedent: government has previously accepted the community's right to comment on this site's forensic operations before they take effect — after, not instead of, two broken assurances.
2015–2025	WA Mental Health, Alcohol and Other Drug Services Plan 2015–2025 published by the Mental Health Commission.	The government's own ten-year strategic plan describes the Graylands institutional model as based on a 'Victorian era asylum model' — language indicating the model was regarded as requiring replacement, not expansion. This plan remained government policy throughout

		the period in which the Taskforce (2021–2023) was planning a \$698M expansion on the same site.
2019	State Budget allocates \$3M for Graylands 'planning and decommissioning studies'	Funding directed toward studying site closure and service relocation — consistent with the direction of the Mental Health Plan 2015–2025. No public document has been released showing the outcome of these studies or when the direction changed.
January 2021	Graylands Reconfiguration and Forensic Taskforce (GRAFT) established. Chaired by Hon. Jim McGinty AM (former Minister for Health and Attorney-General). Membership: 6 senior public servants + 1 independent.	Department of Justice was represented on the Taskforce; no community or local government representation confirmed.
2021–2023	CLMI Act 2023 reform process runs concurrently with Graylands expansion planning	Both processes active simultaneously under the same government. Formal inter-departmental coordination between them has not been publicly confirmed.
Oct–Nov 2021	Taskforce recommends northern campus for forensic expansion. Government accepts. Minister Cook informs Parliament.	Core site decision made under old indefinite detention legal regime. No community input at any stage.
November 2022	Government formally endorses expansion following Taskforce's fourth quarterly report	Formal policy commitment. Still no community consultation. CLMI Act reform actively in progress.
March 2023 ★	Infrastructure WA MIPA: business case has 'insufficient information on which to base an investment decision' — six specific concerns	Statutory advisory body raises serious concerns. Business case funded approximately three weeks later.
March 2023 ★	Criminal Law (Mental Impairment) Act 2023 passes the WA Parliament	KEY COLLISION: New legal framework governing every future Graylands patient passes Parliament at the same moment the business case is being funded.
13 April 2023 ★	CLMI Act 2023 receives Royal Assent	New law formally enacted. Community reintegration now a statutory clinical requirement.
April 2023 ★	Government commits \$218.9M in State Budget — approximately 3 weeks after IWA insufficiency finding	Funded despite IWA finding. New law just received Royal Assent. No documented response to IWA concerns released.
April 2023	Liberal Opposition (Libby Mettam MLA) describes announcement as an 'about-face' and 'sudden decision' at odds with WA Mental Health Plan 2015–2025	Contemporary parliamentary record confirms the reversal was sudden and unexplained.
28 June 2023	Ernst & Young contracted to actually develop the Stage 1 business case and finalise costings	Two months after the funding was already provisioned. Tenders WA, contract DoH202311853.
July 2023	GRAFT formally dissolved. Responsibility transferred to NMHS and Office of Major Infrastructure Delivery (OMID).	Taskforce concludes without public release of its full recommendations or site-selection methodology.
1 September 2023	CLMI Act 2023 comes into full legal force.	All custody orders subject to limiting terms. CSOs available. MIRT can release without AG approval. Safe reintegration is now an explicit statutory purpose, though leave remains case-by-case.
Late 2024	Stage 1 Business Case refined by NMHS and OMID	Whether this refinement fully integrated CLMI Act implications has not been publicly confirmed.

2024–2025	An updated business case for the Graylands Campus Project is approved, securing additional funding	First public confirmation — via a 2026 government tender — that the business case was revisited after the CLMI Act commenced. The specific date has not been published.
January 2025	PDP-phase specialist contracts awarded, including a dedicated security consultancy (Security Consulting Group Pty Ltd, \$64,152)	First documented evidence of security-specific work being commissioned. Findings not publicly released. Tenders WA, contract FINW0623724.
November 2025 ★	Cabinet approves a Project Definition Plan (PDP) for the Graylands Campus Project	Government's own tender describes the PDP as refining the project's 'strategic justification and objectives' and delivering 'value for money' — a scope-and-cost exercise. Nothing published from it addresses site suitability, security, or the surrounding community.
19 Feb–4 Mar 2026	ECI Request for Offer for the Graylands Campus Project released and closed on Tenders WA (Dept of Transport and Major Infrastructure)	Procurement proceeds on the timetable the PDP set, ahead of the public ADCO announcement below.
9 April 2026	Government publicly announces ADCO Constructors' engagement under an Early Contractor Involvement (ECI) contract (the contract itself was awarded 23 March 2026)	Design phase commences. Project enters pre-construction stage.
11 June 2026	Turner & Townsend awarded a cost-consultancy contract covering oversight of the project's "ECI and D&C Phases"	Running to 30 April 2031 — the first public reference to a Design & Construct phase. Tenders WA, contract OMID2026074.
14 May 2026	Minister Carey confirms design phase in Ministerial Statement to Legislative Assembly	First formal parliamentary confirmation. Community still not consulted.
2026–2027 (scheduled)	CTRC relocation to 9 John XXIII Avenue — Government's stated aim (per April 2026 statement) is to complete the transition during 2027, ahead of the main site works.	Works became visible in September–October 2026; the government confirmed to Parliament on 15 September 2026 that completion is targeted for mid-2027, with the campus's main works expected to follow it. The original EPBC referral (September 2025) had anticipated a mid-2026 start — the shift to mid-2027 indicates schedule slippage on the one component of the project closest to a school.
16–17 June 2026	Community writes formally to Sandra Brewer MLA and six Ministers. Acknowledgement requested within 14 days; substantive response within 28 days.	Formal community correspondence on record. Ministerial responses pending.
August 2026	Community research paper published; community petition in preparation	Research and advocacy active and building.
26 August 2026	NMHS awards a \$149,215 contract to develop a business case "to request funding to operationalise" the finished facility	A separate process from the capital business case above, covering recurrent/operating funding. Due to report by April 2027. Tenders WA, contract NMHS202616608.
September 2026	Enabling works become visible at the John XXIII Avenue boundary: HV, communications, water and fire main upgrades under the Early Works Agreement	First on-the-ground activity residents have observed, consistent with the interdependency works described in the May 2026 tender.
September 2026	OMID reissues its project fact sheet, adding for the first time the question	Answer confines mandatory consultation to internal clinical and design input;

	"What consultation is required?"	describes public consultation as something that may occur incidentally. See Section 8.3.
15 September 2026	All 14 of Sandra Brewer MLA's Questions on Notice, lodged 11–13 August 2026, answered on the same day.	Confirms demand modelling accounted for the CLMI Act and DOJ governance involvement; supplies a full bed-capacity table to 2029; confirms mid-2027 CTSC completion with main works to follow. Does not name the basis for the investment decision against IWA's finding, confirm the Project Definition Plan's existence or location, or describe current or planned security protocols (Sections 5.4, 6.2, 12.6).

★ These events occurring within 4–6 weeks of each other in March–April 2023 represent the central governance concern: the government's own advisor found the business case insufficient, the new legal framework governing every future Graylands patient was enacted, and a \$218.9M commitment was made — without any public documentation of how these events were sequenced, coordinated, or reconciled.

Annex C — Key Terms Glossary Term

Definition

CLMI Act

Criminal Law (Mental Impairment) Act 2023 (WA). Replaced the old Criminal Law (Mentally Impaired Accused) Act 1996. In force from 1 September 2024.

Community Supervision Order (CSO)

Under the CLMI Act, a court order requiring a forensic patient to live in the community under supervision, rather than being detained.

Early Contractor Involvement (ECI)

A procurement model in which a contractor is engaged during the design phase to provide cost and buildability input before a full construction contract is awarded.

EPAC Committee

The Standing Committee on Environment and Public Affairs of the WA Legislative Council. All LC ePetitions are automatically referred to this Committee for consideration and report.

EPBC Act

Environment Protection and Biodiversity Conservation Act 1999 (Commonwealth). Australia's primary federal environmental law. Protects nationally threatened species and ecological communities.

Forensic mental health patient

A person detained under a court or tribunal order in the context of criminal proceedings. Forensic mental health is distinct from general psychiatric care — patients are detained because of serious offending. The presence of mental illness does not remove the underlying risk.

GRAFT

Graylands Reconfiguration and Forensic Taskforce. Government-established advisory body, January 2021 – July 2023.

Heritage Impact Statement (HIS)

A formal assessment document required before significant works on a State Heritage-listed property. Must be submitted to the Department of Planning, Lands and Heritage. Triggers a public consultation process.

Infrastructure WA (IWA)

The State's statutory independent infrastructure advisory body, established under the Infrastructure WA Act 2019. Provides independent assessments of major public infrastructure proposals.

Mental Impairment Review Tribunal (MIRT)

The new tribunal established under the CLMI Act 2023 with the power to vary or revoke custody orders — including releasing patients — without requiring the Attorney-General's approval.

MIPA

Major Infrastructure Proposal Assessment. Infrastructure WA's formal assessment process for proposals above a threshold capital value.

NMHS

North Metropolitan Health Service. The WA health service responsible for operating Graylands Hospital and State Forensic Mental Health Services.

OMID

Office of Major Infrastructure Delivery. The WA Government office responsible for delivering major health infrastructure projects.

Questions on Notice (QON)

Written questions submitted by a Member of Parliament to a Minister, requiring a formal written response on the parliamentary record. Responses are published in Hansard.

Sub-acute forensic

A lower security classification for forensic patients who remain under custody orders but are at a recovery stage where maximum-security containment is not clinically required. Includes participation in structured community access programs.

Annex D — Sources and References All sources are primary government documents, statutory bodies, or credible mainstream media. No social media, unverified accounts, or opinion sources have been used. All were accessible as at August 2026. Links are provided for direct verification.

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DISCLAIMER This paper has been prepared for community information purposes using publicly available sources as at August 2026. It does not constitute legal, planning, or medical advice. The community does not dispute the state's legitimate clinical need for expanded forensic mental health services. This paper asserts that the community's right to be consulted, informed, and protected must be met alongside that clinical need — not treated as secondary to it.

Annex E — Frequently Asked Questions This annex addresses the questions most commonly raised about the Graylands Campus Project — including the hard ones. All answers are consistent with the analysis in the main paper and grounded in publicly available sources.

Understanding the Project Q: What exactly is being proposed at Graylands? The State Government is converting Graylands Hospital from a general psychiatric teaching hospital into WA's only large-scale dedicated forensic mental health campus. Stage 1 — currently in design — adds 40 new forensic beds to the northern campus at a committed cost of \$218.9M. The long-term plan, confirmed in the Infrastructure WA MIPA (March 2023), is 136 forensic beds at an estimated total cost of \$698 million. The general psychiatric hospital function will be substantially closed or relocated. This is a change in what Graylands is, not merely an extension of what it does. Q: How is this different from what is already there? Graylands currently holds 45 forensic patients in the Frankland Centre (maximum-security) and Dryandra Ward under the old indefinite detention regime. Stage 1 adds 32 sub-acute beds — a lower-security classification designed for community access programs — and WA's first 8-bed child and adolescent forensic unit. The CLMI Act 2023, in force since September 2024, replaced indefinite detention with fixed limiting terms and made safe community reintegration an explicit statutory purpose, materially expanding the legal mechanisms for supervised leave. The campus is not being expanded within the old model. It is being built into a fundamentally different legal framework. Q: What does 'forensic mental health patient' mean? Are these patients dangerous? Forensic mental health patients are individuals charged with serious criminal offences — including homicide, manslaughter, grievous bodily harm, and serious sexual offences — who have been found by a court either unfit to stand trial or not criminally responsible due to mental illness at the time of the offence. They are detained under court orders, not health admissions. The presence of mental illness does not remove the risk associated with their offending history. That is not stigma — it is clinical and legal fact that responsible government must plan around. The community is not asserting every patient is dangerous in every situation. It is asserting that the government has not publicly disclosed the risk assessment for this campus or the community access program boundaries under the new law.

About Stage 1 and Timing Q: The contractor is already engaged — isn't it too late? No. ADCO Constructions was engaged in April 2026 under an Early Contractor Involvement (ECI) contract — a design-phase mechanism, not a construction contract. Main campus construction has not commenced. The Heritage Impact Statement has not been submitted. The City of Nedlands Development Application has not been lodged. The full construction contract has not been signed. ECI is specifically the phase in which scope and cost are refined before commitment to build. Characterising it as irreversible inverts the logic of the mechanism. Q: Stage 2 is years away and may never happen — why not just accept Stage 1? Stage 1 already introduces categorical changes, not incremental ones. The 32 new sub-acute beds carry a lower security classification built around structured community access under the CLMI Act's expanded leave and reintegration mechanisms. The 8-bed child and adolescent unit is a genuinely new institution being built 270 metres from John XXIII College. Stage 1 nearly doubles the forensic bed count. Under the CLMI Act's higher-throughput model, 85 beds involves substantially more patient-community interaction than 45 beds under the old indefinite detention law. None of the eight documented concerns in this paper are resolved by accepting Stage 1 while waiting for Stage 2. They apply to Stage 1 directly.

Q: Hasn't most of the money already been spent? No. The \$218.9M is a budget allocation from 2023, not spent construction expenditure. As at August 2026, no physical construction has commenced on the main campus or at the CTRC relocation site — the Government's own April 2026 statement put the target for completing the CTRC relocation at 2027, with contractor procurement for that component still underway. A design contract and a not-yet-commenced relocation are not grounds for proceeding to full construction without addressing the documented governance gaps.

About Governance and Process Q: The government says it conducted community consultation — who did it actually consult? The Graylands Reconfiguration and Forensic Taskforce (2021–2023) consulted clinical professionals, mental health consumers, and senior public servants about the best model for delivering forensic mental health services. This was clinical consultation — not community consultation about the impact on surrounding residents. No record of any public meeting with Mount Claremont residents, school communities, or local government exists in publicly available documents. Notification to the City of Nedlands through an EPBC referral — which received no specific response — is the only identified community engagement. Q: Does the CLMI Act really change anything in practice? Yes, materially. Under the old law, ~56 patients were held indefinitely with no end date; release required the Attorney-General's personal approval; the population was stable and long-stay. Under the CLMI Act 2023 (in force September 2024): all orders must have end dates; some retrospective limiting terms had already expired, producing 'unplanned discharges'; courts can now order patients to live in the community under Community Supervision Orders; and safe reintegration is an explicit statutory purpose. A campus under this framework involves fundamentally more patient-community interaction than one under indefinite detention. Q: Infrastructure WA is only advisory — can't the government just proceed anyway? Yes. The MIPA process is advisory, not binding. The community has never claimed otherwise. The argument is that a government that overrides its own statutory independent advisor has an elevated accountability obligation — to show what work addressed the concerns raised, and to release that documentation. The Government committed \$218.9M approximately three weeks after the IWA finding of insufficiency. That documentation has not been released.

Challenging Questions Q: Isn't this just NIMBYism? NIMBYism means objecting to something based primarily on location preference, regardless of evidence. That is not this campaign's position. The documented concerns here are grounded in the government's own sources: the government's own advisory body found the business case insufficient; the law governing every future patient changed in the same month as the funding commitment; the community was never consulted across five years and \$698M of planning; and the prior publicly stated direction was reversed without explanation. These concerns would be equally valid wherever this project was located. The location — a residential suburb 270 metres from a school — is relevant to the community's stake in getting answers, not to the validity of the questions. Q: Doesn't opposing this stigmatise mental health patients? No. The community explicitly and consistently supports forensic mental health services and the need to provide them properly. The concerns raised are about transparency, evidence quality, and community consultation — not about patients or their treatment. Making clinical, legal, and factual distinctions between forensic and general psychiatric patients is not stigma. It is accurate. Refusing to make those distinctions to avoid difficult planning questions does not serve patients, the community, or the quality of the facility being designed.

Q: Graylands has been there for over a century — why is this suddenly a problem? Three things have changed. First, the function of the campus is changing from a general psychiatric hospital with a contained forensic wing to a dedicated forensic campus — a categorically different institution. Second, the CLMI Act 2023 now mandates that patients be progressively integrated into the surrounding community — a clinical framework that did not exist under the old law. Third, the scale is nearly tripling. The community has not objected to Graylands' presence. It is objecting to a \$698M transformation of what Graylands is — made without consultation, without adequate evidence, and without public explanation. Q: Won't this kind of opposition make it harder to build forensic mental health services anywhere? The community's position has been consistent: WA needs properly resourced forensic mental health services. The question is whether this site, this process, and this standard of evidence are adequate. An independent alternative sites analysis — which the community is asking for — would assess whether there is a better location. If there is not, and if the evidence base and community engagement are completed properly, the community will have been given the information it is currently being denied. Opposing inadequate process is not the same as opposing the facility.

What Happens Next Q: What would genuine community consultation look like? At minimum: formally advertised public information sessions in Mount Claremont, covering the full 136-bed long-term plan across every stage — not only Stage 1; direct written notification to every household within a defined radius and to every nearby school community; a genuine opportunity to ask questions and receive answers before the design is finalised. This is not an extraordinary standard. It is the standard applied to major infrastructure projects that materially change the character of a residential area. Q: What specifically is the community asking for? Ten specific asks, summarised: (1) Pause all site works until consultation is completed; (2) Release the full business case and evidence base; (3) Release the decision-making trail behind each key commitment; (4) Show how planning was updated for the CLMI Act 2023; (5) Confirm what the Department of Education was told; (6) Release school consultation records; (7) Commission an independent alternative sites analysis; (8) Publish the community risk assessment under the new law; (9) Publish the boundaries of supervised leave and community access programs near this campus; (10) Hold genuine community consultation before the design is finalised. Full detail in Section 11 of this paper. Q: What can I do? Join the Mount Claremont Community Network's mailing list for updates, and sign the community petition once it is released. Write to the Member for Cottesloe and your federal Member for Curtin. Write to the Heritage Council of Western Australia about the State Heritage-listed campus — this process remains open, unlike the now-closed EPBC referral. Prepare a formal submission when the City of Nedlands Development Application is lodged — a statutory right. Ask your school P&C to write formally to the Ministers. Share this paper with others who should know about it. Contact the Mount Claremont Community Network at mtclaremontcommunity@gmail.com.